



HIGHER HEALTH

Higher Education & Training: Health, Wellness and Development Centre

Improving wellbeing. Inspiring success.



2021/22 ANNUAL REPORT



higher education
& training
Department:
Higher Education and Training
REPUBLIC OF SOUTH AFRICA



health
Department:
Health
REPUBLIC OF SOUTH AFRICA



Health and Welfare Sector
Education and Training Authority
HWSETA



Co-funded by
the European Union



**THE
GLOBAL
FUND**



UNIVERSITIES
SOUTH AFRICA



SAPCO
South African
Public Colleges Organisation

Published by:

Higher Education and Training Health, Wellness and Development Centre
trading as HIGHER HEALTH

October 2022

Company registration number: 2017/419731/08

Address: Wild Olive Building, Riverfalls Office Park,
262 Rose Avenue, Centurion, Pretoria

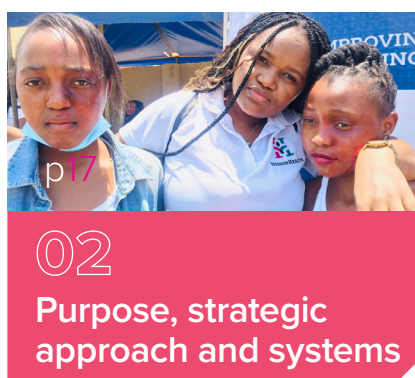
Tel: 011 030 0113

CONTENTS

- 2 Chairperson's Message
- 4 Deputy Chairperson's Message



- 7 Inventory of year's achievements
- 8 COVID-19: the response evolves
- 9 Taking the HIGHER HEALTH Model to scale
- 10 An overview of our main focus areas
- 12 Strengthening health service infrastructure
- 13 Multipurpose communication
- 13 New frontiers: the CET sub-sector
- 13 Sustaining and forming partnerships
- 14 Growing our human capacity
- 15 Growth in revenue and spending
- 16 Looking ahead
- 16 Acknowledgments



- 19 Strategic approach
- 21 University and college partnerships
- 22 Student volunteers as a driving force
- 22 Structures for management and delivery



- 24 Enabling the sector's COVID-19 response
- 26 Sexual and reproductive health programme
- 30 Mental health programme
- 32 Gender-based violence programme
- 34 Programmes to support vulnerable populations
- 36 University programme
- 37 Community education and training programme
- 38 Communication programme



- 48 General information
- 49 Audit Committee report
- 50 Directors' responsibilities and approval
- 51 Directors' report
- 52 Independent auditor's report
- 54 Extracts from annual financial statements

CHAIRPERSON'S MESSAGE



In 2020, we were under the impression that meeting the demands of the COVID-19 pandemic would be a sprint. By 2021, we had to concede that we were participants in an ultramarathon and this required a different set of capabilities and type of fitness. HIGHER HEALTH has proved well able to adapt and incorporate the sustained COVID effort into its programme of activities.



It is now clear that the impact of the pandemic on the post-school education and training (PSET) sector will be felt long after infections have subsided to endemic levels. The extensive loss of contact teaching in their final year of schooling took its toll on students entering universities and colleges in 2021. Many were not ready for the kind of learning required of them and this inevitably impacts on their wellbeing and possibly their health. In this context, HIGHER HEALTH's significance as a support to academic success is arguably greater than ever.

HIGHER HEALTH itself is in robust condition. The growth in the scope of its work and responsibility has, fortunately, been matched by an increase in funding. It is to HIGHER HEALTH's credit that both local and global funding organisations view it as an effective and trustworthy partner. However, there are both benefits and pitfalls to being the "go to" organisation for on-the-ground work in the PSET sector. Every opportunity to expand should be carefully weighed against HIGHER HEALTH's mandate to ensure we remain strategically focused and retain our characteristic nimbleness.

During 2021/22, COVID-19 continued to be a major feature of campus life. The constraints placed on contact teaching by infection control requirements were considerable. With lecture halls and classrooms operating at only 50% of capacity, hybrid learning was the only viable option for many institutions. Especially in the university sub-sector, this was a limitation to HIGHER HEALTH's programme of activities which rely, to a great extent, on the power of direct, interpersonal engagement.

As the option of mass vaccination approached, **HIGHER HEALTH played a key role in ensuring students and staff at every institution had convenient access to a vaccination facility.** The organisation also championed vaccination consistently and loudly but did not weigh in on the matter of mandatory vaccination, which evoked both reasoned debate and heated contestation at PSET institutions. Some institutions opted for vaccine mandates while others chose to drive demand for vaccination through education and persuasion.

The combined mental health impacts of COVID-19 itself and the interruption of contact learning have been a major concern over the past two years. Perhaps the effect on staff members has been underappreciated, while the psychosocial consequences for students have been more closely

investigated. During 2021/22 researchers from several universities surveyed some 80 000 students (results may be available before this report appears) and a key finding was that students are, for the most part, very resilient. We need to harness that resilience by fully opening up institutions, recreating vibrant campus communities, and tackling COVID-linked academic deficits with determination.

HIGHER HEALTH's programming is founded on the reality that health is determined not only by biology but is profoundly shaped by psychological, social and economic factors. **We strive to empower young people to make choices that preserve their health and promote their wellbeing,** and we combat the stigma and discrimination that erode wellness. It is no accident that HIGHER HEALTH's focus areas include gender-based violence, LGBTQI+ health, and the health of people with disabilities.

However, a few times each year we are reminded that we need more and much stronger interventions of this kind as we witness appalling acts of racism and misogyny on our campuses. These acts of cruelty – and the shadowy threats of such cruelty – are damaging to the wellbeing of students as individuals and a community. There is a compelling argument for an extended and immersive "shared humanities" experience for all students to give them a chance to absorb the personal values and qualities needed to build a unified society. It is possible that HIGHER HEALTH could play a role in creating such a transformational rite of student passage.

As my review of the past year suggests, the Board held important strategy discussions during the year. It also responded to the changing oversight needs of the organisation in light of its exponential growth over the past two years. We have attempted to preserve the agility of the organisation – which is part of its value to partners – and have ensured that monitoring and accountability keep pace with expansion.

While the service output from HIGHER HEALTH's partnership with the university sub-sector was constrained during 2021/22, I believe that institutions in the sector value HIGHER HEALTH more greatly than ever. We realise that health issues may constitute a major threat to higher learning and research and that the unified response that HIGHER HEALTH enables is more effective and efficient than our individual efforts would be.

Prof Wim de Villiers
Chairperson

DEPUTY CHAIRPERSON'S MESSAGE



In the continued presence of COVID-19, technical and vocational education and training (TVET) colleges returned partially to face-to-face learning and teaching in the latter part of 2021 and resumed classroom activities fully in early 2022. HIGHER HEALTH played a critical role in building the confidence of college management so that normal functioning of their institutions could be reinstated safely.



Over the past two years, the relationship between the TVET sub-sector and HIGHER HEALTH has been redefined. Not only have institutions come to appreciate how health and safety issues impact on the effective delivery of education programmes, but they recognise the leadership of HIGHER HEALTH in identifying and responding to key health issues.

The value of HIGHER HEALTH during this anxious and confusing time has been amazing. Having clear COVID-19 guidelines for the sector and assistance at college level for implementing guidelines and protocols, really helped colleges to move forward. Prior to the pandemic HIGHER HEALTH was seen as a programme that benefited students. Now **HIGHER HEALTH is viewed as a resource for everyone in the TVET sub-sector**. College staff greatly appreciated having early access to vaccination and college principals could see how this reduced their staff members' fear of infection.

The figures in this report reflect a high level of student participation in HIGHER HEALTH programmes at TVET colleges. Students are very willing to attend Second Curriculum sessions, complete assessments of their individual health risks, and volunteer as peer educators. I put this down to HIGHER HEALTH's visibility on campus through the hiring and placement of campus health and wellness coordinators (CHWCs).

It is true that COVID-19 temporarily disrupted HIGHER HEALTH's broader programme of activities. Serious matters, like HIV prevention, were put on the back burner while we geared up to combat COVID-19. Many of our partners in the healthcare field were affected in a similar way. In 2021, we saw a strong recovery in the scope of health promotion via health days and other HIGHER HEALTH activities. However, many clinic services at TVET colleges that depended on partnerships with district health services closed down as COVID arrived and have yet to resume operation. The re-establishment of clinics is a priority for the future and we believe that the sound relationships we forged with health authorities in the COVID-19 response will make this task easier.

In contrast, we have seen progress in mental health services and responsiveness to gender-based violence (GBV) in the past year. The lockdown conditions imposed in the early months of the COVID-19 pandemic triggered mental health crises for many students and appeared to increase their exposure to GBV. HIGHER HEALTH incorporated services in these areas into its COVID-19 response and we have continued to feel the benefit. GBV, in particular, is a complex matter involving not only health and psychosocial support, but also knowledge of legal and disciplinary issues. Once again, **HIGHER HEALTH has guided colleges on how to respond to GBV and the related issue of sexual harassment** of staff and students.

It seems to me that **HIGHER HEALTH has also laid the foundation for other organisations that are keen to collaborate with TVET colleges in the area of youth development**. In the past year, the Independent Electoral Authority (IEC) worked with our colleges to increase civic awareness among young people. This underscores the value of HIGHER HEALTH's concept of the Second Curriculum that prepares students for life even as the primary curriculum prepares them for work.

I am proud to be associated with HIGHER HEALTH, particularly at a time when external challenges have only strengthened its leadership and influence in the field of student health, wellness and development. I admire the vision of Prof Ramneek Ahluwalia, in his role as CEO, and commend the staff for their energetic translation of this vision into reality. I also thank my fellow Board members, and particularly our Chairperson, Prof Wim de Villiers, for contributing to the growth of this invaluable resource in the field of higher education and training.

Sanele Mlotshwa
Deputy Chairperson

CEO'S OVERVIEW

01



In the past year, the COVID-19 pandemic remained a compelling priority for universities and colleges in the post-school education and training (PSET) sector. The organisation of a national network of vaccination sites for students and staff was certainly the greatest new challenge HIGHER HEALTH assumed during the year.

But, unlike in 2020, COVID-19 did not overwhelm other aspects of HIGHER HEALTH's programme of student health, wellness and development. On many campuses our interventions began to reconstitute as a comprehensive youth health and wellness programme. In some areas of work we experienced great resilience, and service outputs actually exceeded our pre-pandemic performance.

The past year also saw the systematic extension of HIGHER HEALTH's support to the community education and training (CET) sub-sector. This had long been our intention and, after the COVID-19 crisis thrust us into working with CET colleges in 2020, we found ways to establish a more sustainable relationship in 2021/22.

However, the ability of universities and colleges to implement the HIGHER HEALTH Model of healthcare depended to a large degree on the physical presence of students on campus. TVET colleges returned fairly significantly to contact learning and teaching in 2021, while most universities maintained a hybrid model with students on campus intermittently. This difference was clearly reflected in our programme outputs for 2021. The 2022 academic years saw further opening up in both sub-sectors.

In rounding up my opening comments, I must acknowledge the outstanding collaboration and the many partnerships that continue to be the force that propels our work. The HIGHER HEALTH programme is a glorious patchwork (with a clear pattern) produced by a large number of diverse role-players, motivated in different ways, with diverse skills and resources, but all ultimately joined by a desire to give young people a better chance in life. Thank you all for stepping up to play your part.

INVENTORY OF YEAR'S ACHIEVEMENTS



COVID-19 management

Coordination of access to vaccination services through:

- 19 accredited campus vaccination sites;
- 57 campus vaccination points; and
- Links to 4 000 community or workplace vaccination sites.

An estimated two-thirds of two million students and staff were fully or partly vaccinated by 31 March 2022.

Some 18.8 million COVID-19 "passports" to campus had been issued to students and staff by the digital screening app since its launch in 2020.



Knowledge transfer for health

- More than 115 000 students enrolled for Second Curriculum educational sessions on HIV, TB and STIs, and more than 107 000 for sessions on mental health and GBV.
- We trained 1 621 peer educators to deliver peer-to-peer activities.
- Health programmes were broadcast by 20 campus and community radio stations with a combined audience of 1.1 million who were reached on a weekly basis.
- Institutions held a total of 2 768 campus health days which offered information and screening opportunities.



Student actions for health

Nearly 65 000 students completed self-assessments of their risk of acquiring HIV, other STIs or TB and nearly 70 000 assessed their own mental health status and vulnerability to GBV.



Access to health products and services

- HIV tests were administered to about 98 600 individuals, while 105 700 underwent symptom screening for TB and 108 300 for an STI.
- Nearly 55 000 students were screened for breast, cervical or prostate cancer.
- A total of 11 210 students called or messaged the 24-hour helpline for crisis counselling or information.
- About 6.34 million condoms were distributed.
- Some 64 500 other contraceptives were supplied.
- HIGHER HEALTH psychologists had 6 221 individual or group counselling sessions with students.
- Campus mental health and counselling services assisted 25 964 students.

COVID-19: THE RESPONSE EVOLVES

IN 2020/21, the Minister of Higher Education, Science and Technology, Dr Blade Nzimande, entrusted HIGHER HEALTH with the task of providing guidance to the PSET sector on COVID-19 and enabling universities and colleges to implement guidelines and protocols effectively. This was an enormous responsibility, but HIGHER HEALTH's established track record of health programme development in the PSET sector stood it in good stead. We could draw on established relationships with institutional management, staff at the service level and an existing corps of student volunteers trained in the techniques of peer-to-peer education.

Our initial focus in the first year of the pandemic was developing guidelines and protocols, providing training to a wide range of implementers, conducting mass communication on COVID-19, establishing screening stations, and providing personal protective equipment to institutions. Slightly later in 2020 HIGHER HEALTH conducted research on the impact of COVID-19 restrictions on students, created the HealthCheck COVID-19 screening app for multiple platforms, and began to respond to the psychosocial needs of students. This we did by providing a mental health guideline to institutions, developing the capacity of staff to assist students, establishing a toll-free helpline, and deploying psychologists and mobile clinics to rural and semi-urban campuses that lacked access to facilities.

IN 2021/22, the pandemic environment changed and, with it, HIGHER HEALTH's focus. Vaccination shifted firmly to the centre of our vision and we came to understand the wave-like pattern of infections, linked to the periodic emergence of new variants of the virus. Our role in this climate was:

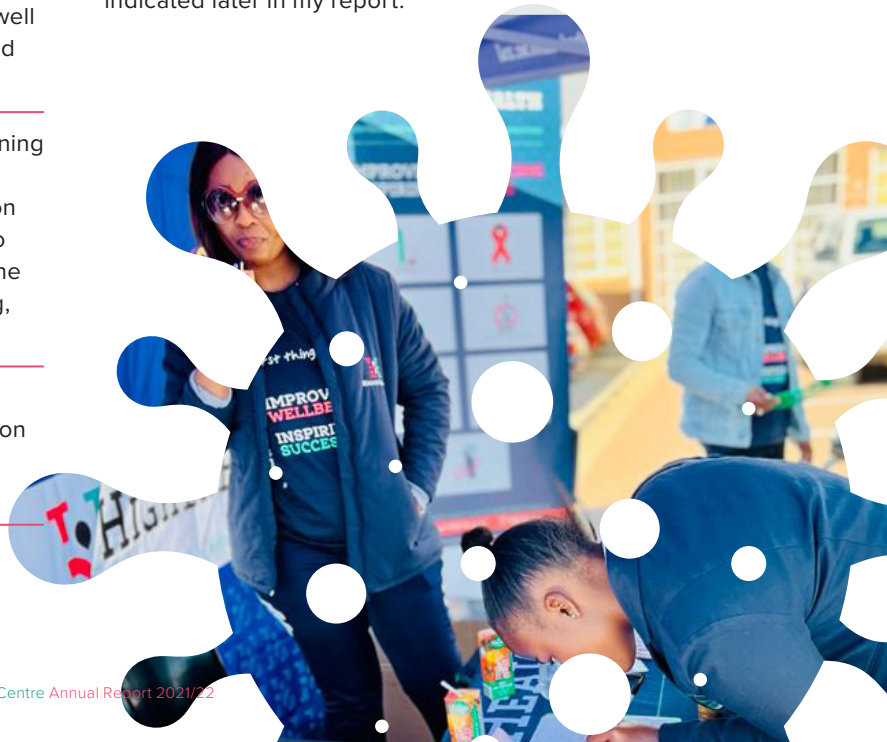
- Ensuring that students and staff at every college and university had **convenient access to a vaccination site**. This involved facilitating the setting up of dedicated vaccination sites and points on larger campuses as well as linking smaller campuses to sites run by public and private healthcare organisations.
- Providing fresh guidance on social distancing in learning and teaching venues and places of residence as the return to campuses gained ground and constraints on accommodating students became apparent. We also continued to **reinforce the behaviours** that reduce the spread of COVID-19: mask wearing, social distancing, hand washing and self-isolation after exposure.
- Developing a **draft vaccination policy** for the sector. This was submitted to the Ministry of Higher Education and Training early in 2022 and will be reviewed and amended as necessary before publication.

Communication in various forms – consultation, information-sharing and advocacy – was the common thread linking all our COVID-19 interventions. The coordination of the PSET sector vaccination network involved extensive consultation with stakeholders on the demand and supply sides.

As the discourse on COVID-19 vaccination became more polarised and divorced from the evidence, and fatigue set in regarding mask-wearing and social distancing, communication became more crucial than ever. HIGHER HEALTH continued to advocate without compromise for proven public health measures, including vaccination, and encouraged institutions to do likewise.

However, when it came to drafting the vaccination policy, HIGHER HEALTH listened to a variety of positions on human rights and vaccine mandates. We thought it was important for the credibility of the document to consult honestly and widely.

In light of the previous year's research on the psychosocial impact of COVID-19 on students (which we conducted in partnership with the Human Sciences Research Council), we further expanded our efforts to address gender-based violence and mental health, as indicated later in my report.



TAKING THE HIGHER HEALTH MODEL TO SCALE

The HIGHER HEALTH Model was substantially overhauled in 2019 and the human resource machinery required to power the new model – especially at TVET colleges – was primed and ready to go early in 2020. And then COVID-19 descended. It was 2021 before circumstances in the TVET sub-sector approximated “normal” and the model got a fair trial run and showed its potential.

HIGHER HEALTH prioritises the prevention, early detection and early management of illness and promotion of general wellbeing. We invest in health education and empowerment of students to take responsibility for their health. The HIGHER HEALTH Model, explained more fully on page 19 and 20, sets out three basic levels of intervention.

LEVEL 1

KNOWLEDGE TRANSFER – which includes a range of health awareness, health promotion and structured health education opportunities. The Second Curriculum and a range of peer-to-peer interventions are key elements of this level of intervention.

LEVEL 2

PRACTICAL LEARNING – which advances health-seeking by encouraging students to use a screening tool to assess their personal risk in relation to HIV, TB and sexual health more broadly, mental wellbeing and exposure to gender-based violence (GBV) and harmful use of alcohol and drugs.

LEVEL 3

LINKAGE TO SERVICES – which requires that appropriate health services are easily accessible to students, preferably right on campus but, where necessary, by referral to community facilities. HIGHER HEALTH provides some of these services directly, facilitates the extension of public health services to campus settings, and enhances clinics run by colleges and universities. Services also include the toll-free 24-hour helpline available for students experiencing psychosocial crises.

The mobilisation of student participation in health activities rests on the shoulders of full-time campus health and wellness coordinators (CHWCs) and peer educators, who are students acting in a voluntary capacity. CHWCs are either employed by universities or by HIGHER HEALTH. In the latter cases, they are seconded to individual TVET colleges and CET centres.

In the 2021 academic year, in the TVET sub-sector, the number of First Things First health days organised and the number of students who enrolled in the Second Curriculum and completed self-assessments of health risk exceeded figures for the previous year and 2019. The number of HIV tests, and TB and STI screenings performed exceeded 2020, but did not quite recover to pre-COVID levels. The figures suggest that the strategy of integrating First Things First activities with COVID-19 screening stations was effective.

At universities, there was generally a modest increase in 2021 in participation in all the above activities relative to 2020. However, activity did not approach pre-COVID levels. The limited return to contact teaching and learning not only reduced the general pool of students on campus on any day but also affected the availability of peer educators who play a crucial role in mobilising participation.

Across the sector, a considerable number of students identified themselves as being at high risk in terms of sexual health, mental health or GBV. Among students who completed the HIV, TB and STI self-assessment, 16% scored in the high-risk zone, while the high-risk rate for mental health was 13% and GBV 8%. Realistic risk perception plays a significant role in motivating individuals to seek professional assistance and many of the high-risk students accepted a referral to an appropriate service.

AN OVERVIEW OF OUR MAIN FOCUS AREAS

HIGHER HEALTH prioritises particular areas of health and psychosocial care based on evidence that they are especially relevant to young people. In the past two years, of course, COVID-19 was added to the top of the list and I have already reviewed activities in this regard. A quick overview of the other areas follows and more detail is provided in Section 3.

Mental health and wellbeing

A growing body of research confirms a high prevalence of mental health problems, such as anxiety and depression, among students and shows that this impacts directly on academic performance. **HIGHER HEALTH first recognised mental health as a PSET priority five years ago and began to break the silence that surrounded the subject and build capacity to assist students.** In 2020, by employing our own team of psychologists, setting up a toll-free 24-hour crisis line in partnership with the SA Depression and Anxiety Group (SADAG,) and enabling our CHWCs to provide first-line support and referral, we began to augment the established campus counselling services, which existed almost exclusively at universities. 2021 reflected further progress, with the highest utilisation of services to date. However, there is still a large unmet need and stigma remains a major barrier to students seeking mental healthcare. HIGHER HEALTH continues to challenge stigma by using multiple communication channels to discuss mental health.

Sexual and reproductive health (including HIV, TB and STIs)

Although the prevention of HIV still receives special attention, this is increasingly in the context of a positive sexual and reproductive health (SRH) programme. We encourage all students to view SRH services as a right and, in the case of women students, as an instrument for equity. In the last year, we were gratified to see a notable increase in demand for long-term contraceptives even on university campuses where the demand for many other services declined. **Our condom distribution gained ground relative to 2020, with 5.26 million male condoms and 1.08 million female condoms provided.** However, there are signs that we cannot be complacent: we treated 6 301 STIs, performed 14 661 pregnancy tests and referred 393 students for termination of pregnancy.

In addition, the decline in HIV testing in the last year needs to be closely monitored to establish whether this was purely due to the absence of students from campuses. The importance of testing, treating and establishing viral suppression among students living with HIV cannot be overemphasised as a tool to reducing new infections as well as maintaining the health of students living with HIV.

Gender-based violence

GBV is arguably the most complex and intractable of our focus areas. It is also a programme that we have had to help build from ground level, as very little had been done in the sector. Over the past few years, much of HIGHER HEALTH's work has been directed to providing sectoral guidance through the drafting of policies and guidelines and the training of relevant campus staff in applying these. There are now signs that our systems are beginning to yield support for affected students: **in 2021, a total of 3 795 students affected by GBV were referred for psychosocial support.** Equally important, our policy and strategic work is beginning to tackle the culture of sexual entitlement that fuels GBV.

Vulnerable groups

The HIGHER HEALTH programme has identified three groups of students who have an elevated risk of compromised health: adolescent girls and young women (AGYW), LGBTQI students, and students with disabilities. Their needs vary but there are certain common features. Stigma and discrimination frequently play a role in placing them in situations where their health is at risk. Health services are often insufficiently responsive, largely because health workers do not understand their specific requirements.

HIGHER HEALTH's three-year grant from the Global Fund to Fight HIV, TB and Malaria for a comprehensive package of care for AGYW concluded in March 2022. The grant focused on preventing HIV infection and unintended pregnancy among woman students at selected TVET colleges and increasing their likelihood of completing their studies. We were funded to provide core HIV prevention services to students and ensure access to other "layered" health and social services through strong referral links. Importantly, the programme held enrolled students within a web of care for multiple years. Although the COVID-19 pandemic sliced through the middle of the programme, over three years HIGHER HEALTH was able to provide HIV prevention services to 42 202 students (78% of the targeted number), enable 27 886 to undergo HIV testing (68% of the targeted number) and link 74 who were newly diagnosed with HIV to care (76% of the targeted number).

We continued to promote the rights of LGBTQI+ students and those with disabilities by educating the broader student body and challenging preconceptions and prejudice. In 2021, a total of 44 773 students enrolled in the Second Curriculum module designed to help build a more supportive environment for these minorities on campus.

Tackling harmful alcohol and drug use

Major surveys conducted earlier by HIGHER HEALTH in the university and TVET college sub-sectors respectively, established that a substantial proportion of students and staff members abused alcohol. They also showed that other drugs were quite commonly used. Our alcohol and drugs intervention aims to reduce substance use – and particularly unsafe use of alcohol and drugs – and minimise the harm associated with such use. Self-screening for the risk of alcohol and drug abuse was introduced in 2021, mainly in Gauteng and KwaZulu-Natal, with referrals for students identified as high risk.



STRENGTHENING HEALTH SERVICE INFRASTRUCTURE

Linkage to appropriate, acceptable and accessible care is a key pillar of the HIGHER HEALTH Model and developing campus clinic services has been a major aspect of the value we have brought to the sector. This has only been possible through close collaboration with provincial health departments, district health services, social services and other agencies that leveraged some of their resources to close gaps on campuses.

In some cases, the province actually established new clinics on TVET campuses. In others district health staff were assigned to run part-time clinics on campuses. In yet others, government's provision of medication and other supplies enabled existing campus clinics to offer a more relevant youth health service.

However, COVID-19 demanded a reallocation of public sector healthcare personnel and resources and, with students away from campus, many of these campus health clinics ceased functioning. At the same time, **HIGHER HEALTH deployed a fleet of 12 mobile clinics to serve TVET campuses on a rotational basis**, focusing on areas with less developed community health facilities.



MULTIPURPOSE COMMUNICATION

Communication is the multitool of programme development and implementation. It is an instrument of education, advocacy, coordination and demand generation. HIGHER HEALTH strives to use a combination of media to communicate in different ways to different audiences. During 2021:

- Our interactions with the country's mainstream media enabled us to reach several million readers, viewers and listeners and raise public awareness of the issues affecting the wellbeing of students.
- Our Future Beats partnership with 20 campus and community radio stations facilitated a continued dialogue on health with some 1.1 million listeners of these stations. The HIGHER HEALTH partnership also enabled the Voice of Wits to produce a series of weekly programmes on COVID-19 which was aired on the other 19 stations and won accolades at the National Radio Awards.
- We organised webinars to provide expert guidance to stakeholders at critical junctures in the course of the COVID-19 pandemic and took full advantage of many opportunities to share information by participating in our partners' webinars and events.
- We provided all campuses with posters and pamphlets to enable them to convey essential health messaging to students.

NEW FRONTIERS: THE CET SUB-SECTOR

IN 2021, HIGHER HEALTH began to extend its model of practice to the CET sub-sector, thanks to dedicated grants from the Department of Higher Education and Training (DHET) and the Health and Welfare Sector Education and Training Authority (HWSETA).

Our initial focus was building the capacity of the nine provincial CET colleges and selected training centres to keep teaching staff and students safe in the face of COVID-19. We also began to lay the foundations for a multifaceted health promotion and preventive health programme that mobilises students as peer educators.

The CET sub-sector is unique. It is highly dispersed, comprising more than 3 000 training centres of varying size, and all its students are studying part-time. Our target during year one was to work with 200 of the larger centres and we managed to activate the programme at 157 centres.

SUSTAINING AND FORMING PARTNERSHIPS

Partnerships are at the heart of HIGHER HEALTH's ability to coordinate a national health programme for the PSET sector. The resources that keep this enterprise on track and moving strongly forward are contributed by a wide range of organisations and the passionate individuals who drive them. Our joint commitment to education and the health and wellbeing of South Africa's young people is the glue that holds us together.

Our oldest partners are the DHET (and its forerunners), Universities South Africa (USAf, and its earlier incarnations) and the country's public universities. Under the leadership of Minister of Higher Education, Science and Technology, Dr Blade Nzimande, and his Deputy Minister, Mr Buti Manamela, our relationship with the department has gone from strength to strength. We are deeply appreciative of the support we receive.

The public universities and TVET colleges – their leadership, key members of staff and student volunteers – are unquestionably our closest and most critical partners. They have walked the journey alongside HIGHER HEALTH and helped build the model that currently guides our joint efforts. The COVID crisis of the past two years has only served to strengthen our partnerships. We look forward to building equally strong links with CET colleges and centres.

Just slightly less significant than the PSET institutions – but still mission-critical – are several categories of partner.

- Public health sector structures – various sections within the national Department of Health, provincial health departments and numerous health districts who collaborate with us on strategic issues and in terms of enhancing service delivery to campuses.
- International and national funding partners who have invested in our programmes and enabled our development as an organisation. Our funders include national government departments and training authorities, major global health funders, members of the United Nations family, national non-governmental organisations, and occasionally private sector companies.
- Research bodies and individual experts who have generously shared their expertise and, by guiding HIGHER HEALTH, have enabled us to play a more valuable role within our sector. This was especially true in relation to the COVID-19 pandemic where HIGHER HEALTH was repeatedly challenged to take on responsibilities beyond its traditional role. We never lacked reliable, highly-informed advice offered with great willingness.

GROWING OUR HUMAN CAPACITY

In terms of human resources, HIGHER HEALTH has experienced two major growth spurts in recent years: in 2019 we established regional and provincial offices and created a network of CHWCs and in 2020 we added psychologists and nurses to our staff complement. In the past year we expanded to support our work in the CET sub-sector and allocated CHWCs to this sub-sector. However, the overall number of staff in the reporting period has remained similar to last year's total.

A striking feature of our human resources profile is that many of our employees are young. Most of our CHWCs are just a few years older than the students with whom they work – and they gain immense experience by performing challenging and responsible activities on a daily basis.

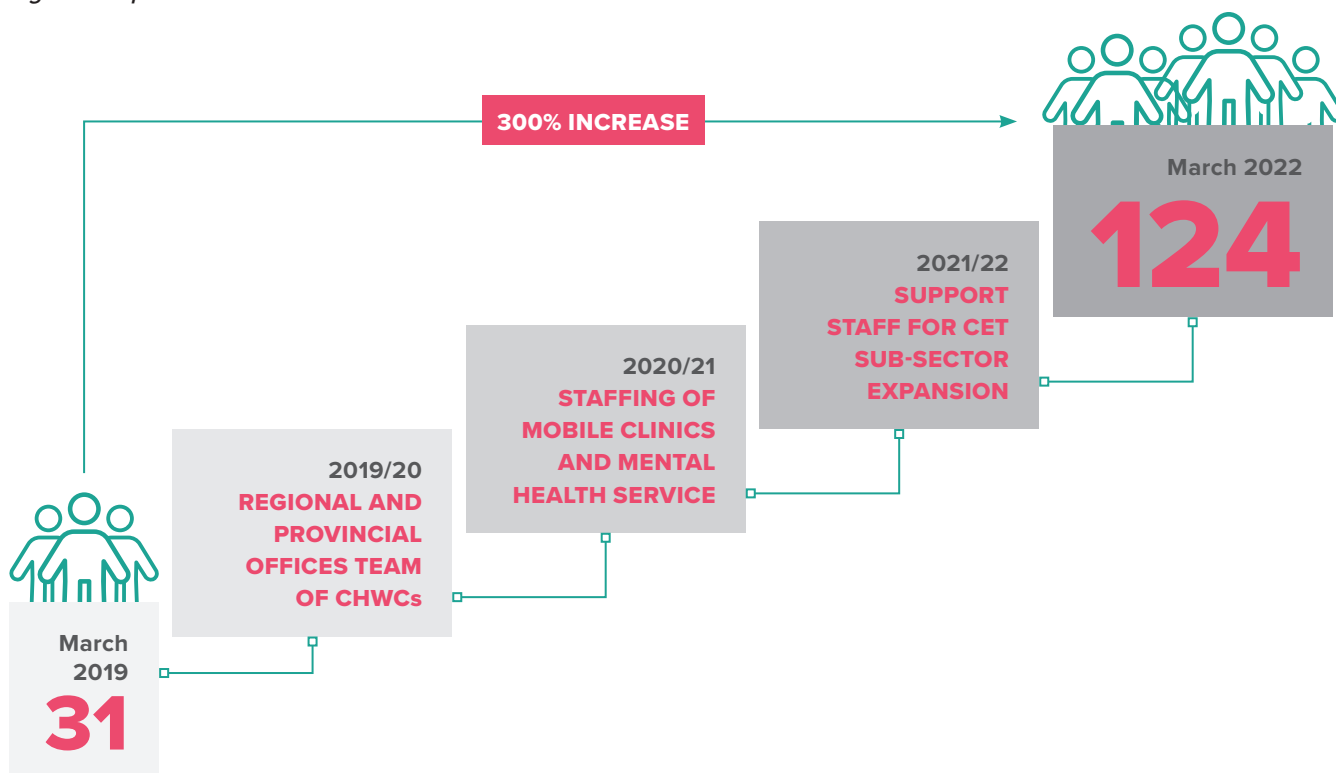
IN MEMORIAM

In the course of the past year, HIGHER HEALTH suffered the loss of valued colleagues: George Chetty, a mainstay of our finance department, passed away just a few months after we lost Communications Officer Simba Honde and a dedicated nurse working in Limpopo, Beauty Sepaela. We remember them with affection and respect.



More detail on HIGHER HEALTH's human resources appears in Section 4.

Figure 1: Expansion of HIGHER HEALTH's human resources



GROWTH IN REVENUE AND SPENDING

As HIGHER HEALTH developed a more comprehensive approach to health, wellness and development and strengthened its capacity to provide support to institutions, the organisation invested an enormous effort in mobilising financial resources to match its vision. In addition, funding was made available from a variety of sources for the responsibility we willingly assumed in relation to the sector's COVID-19 response. The overall result has been a rapid growth in revenue.

In these circumstances it is especially satisfying to report that HIGHER HEALTH has achieved an unqualified audit report for the fifth consecutive year.

In the past four years, HIGHER HEALTH's total income has more than doubled and amounted to

R79.6 million in 2021/22.

The organisation's capacity to utilise available funds has varied only slightly from year to year, with the annual surplus ranging from about **0.3%** of income (in 2018/19) to **6.8%** (in 2019/20). In 2021/22 the surplus amounted to 3.0% of total income.

Further details are contained in excerpts from the annual financial statements at the end of this report.

Since our revenue consists almost exclusively of grant funding, it is governed by various contracts entered into with funding partners and the reporting requirements of these contracts. HIGHER HEALTH appreciates that this accountability not only fulfils our funders' governance requirements but also strengthens our own.

Figure 2: Growth in HIGHER HEALTH annual income and expenditure

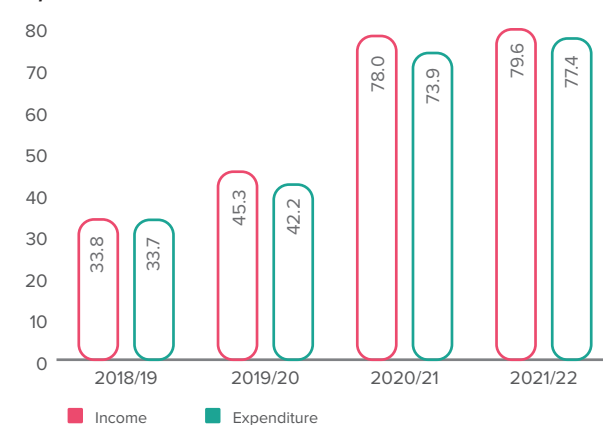


Table 1: Summary of income and expenditure for financial year 2021/22

	2021/22 R million	2020/21 R million
Income		
Revenue – grants	79.82	72.46
Cost of sales	(0.16)	–
Gross profit	79.66	72.46
Other income	(0.01)	5.55
Income	79.65	78.01
Expenditure		
Operating expenses	76.98	74.16
Finance costs	0.27	0.35
Surplus for the year	2.40	3.51

LOOKING AHEAD

I would like to mention just a few priorities for HIGHER HEALTH in the forthcoming year, the first of which is to advance the development of our health and wellness programme in the CET sub-sector. We intend to enlarge our service offering at CET centres where we are already established and bring more centres within our ambit.

We have a strong interest in exploring wider and more creative use of digital platforms to deliver our programme. We recognise that the experience of COVID-19 has changed our lives permanently in terms of how and where we work and study. Blended classroom-digital learning will remain a characteristic of many universities, and HIGHER HEALTH must find the strategies and the applications to take full advantage of the digital platforms that are being developed by institutions. Our utilisation of the HealthCheck digital screening app gave us only a glimpse of what is possible. We intend to build on this experience in collaboration with various institutions.

We will also review some of our management structures and systems in the year ahead. HIGHER HEALTH has made a habit of outgrowing its organograms and we are due for another update. We will also focus on streamlining and automating our monitoring and evaluation systems and incorporating a stronger research component as we gather routine data.

ACKNOWLEDGMENTS

In 2021/22 HIGHER HEALTH remained conscious of our responsibility to honour the trust placed in us to guide the sector's COVID-19 response. As in the previous year, we met with enormous goodwill when we sought advice from experts and we encountered a solution-oriented response from collaborators in major undertakings, like the national vaccination plan for the sector.

It is impossible to acknowledge every contribution individually, but I would like to mention some of our major supporters.

We are grateful to the Minister of Higher Education, Science and Technology, Dr Nzimande, for his confidence in HIGHER HEALTH and his visible support for key programmes. We remain indebted to Deputy Minister Buti Manamela for his sustained and very genuine interest in our work.

We appreciate the leadership of vice-chancellors of universities and principals of TVET and CET colleges in keeping the COVID-19 response on track as campus communities – like the rest of the country – grew weary of the restrictions required. It has been a pleasure to work with leadership so motivated to serve the interests of staff and students.

We are indebted to a large number of role players in the COVID-19 vaccination space for helping us configure vaccination services for staff and students nationwide. They included relevant sections of the national Department of Health, provincial and district health services, private service providers, and PSET institutions across the board. We would like to recognise the role of student leadership, peer educators, and campus and community radio stations in building informed support for vaccination.

We are endlessly grateful to our increasingly diverse group of funders for the resources that enable us to translate our goals into action. We have enjoyed the sustained support of international funding organisations, United Nations agencies, government departments, other public sector structures, and training authorities.

At a service-delivery level, despite the expansion of HIGHER HEALTH's own capacity, campuses depend to a large extent on partnerships with various provincial and district health services as well as non-governmental and community-based organisations. We would like to recognise their invaluable contribution.

Finally, I wish to acknowledge the HIGHER HEALTH family: members of our governance, advisory and operational teams who are at the centre of this enterprise.

The HIGHER HEALTH Board, under the guidance of our chairperson, Prof Wim de Villiers, presides over a dynamic, growing organisation. Continual expansion and absorption of new programme priorities can be a challenge for those who hold the reins of governance. I sincerely appreciate how the Board has kept the balance between embracing opportunities and exercising sober judgment.

I am extremely grateful to the chairs and members of the technical task teams whose intellectual capital is on loan to HIGHER HEALTH. Our organisation's role in developing sectoral strategy frameworks, policies and guidelines has grown considerably and we simply could not manage this without your knowledge, insight and experience.

Finally, to our managers and staff at HIGHER HEALTH. Very few of you have the luxury of a nine-to-five job. Nearly all of you work directly with and for people day in and day out – and know the rewards and challenges of this intense human interaction. I thank you for your dedication and hope that you feel the pride that I feel in the progress we have recorded and the difference we are making in the lives of young people.

Dr (Prof) Ramneek Ahluwalia
Chief Executive Officer



02

Purpose, strategic approach and systems

OUR PURPOSE, STRATEGIC APPROACH AND SYSTEMS

HIGHER HEALTH is a non-profit organisation that enables public universities and colleges in the post-school education and training (PSET) sector to develop and deliver a comprehensive programme of youth health services to their students.

Our collaboration with the sector is driven by the reality that students who are healthy – in the broadest possible sense of the word – are more likely to complete their studies, attain employment that improves their quality of life, and contribute to the security of their families and the development of communities and our nation as a whole.

The organisation has its roots in a government initiative, the Higher Education HIV/AIDS programme (HEAIDS), that was launched more than two decades ago amid fears that the HIV pandemic might decimate universities in South Africa. With the introduction of antiretroviral treatment (ART) for HIV and the rapid expansion of access to treatment, this fear abated. However, the unprecedented focus on student health highlighted:

- The link between student health and wellness, the academic performance of individuals, and the success of higher education institutions in terms of student throughput and quality of graduates.
- The range of health issues that impact on the wellbeing of students, the connections between them, and the logic of tackling campus health as comprehensively as possible.

Another legacy from the HEAIDS era was a special relationship with the ministry and government department responsible for higher education and training, currently the Ministry and Department of Higher Education and Training (DHET). Although HIGHER HEALTH is independent, we continue to serve as a resource to the Ministry and DHET in areas where we have expertise and enjoy strong leadership support for our work.

Currently, the focus areas of the HIGHER HEALTH programme (see box) bear a marked similarity to the main activities and services outlined in the National Youth and Adolescent Health Strategy of 2017. The national strategy did not directly shape our thinking: we walked parallel paths to the same general destination. But both processes were evidence-informed, incorporating formal research and lessons learnt through practice, including interactions with young people. The similarity of their priorities assures us that HIGHER HEALTH has identified the most pertinent health issues of post-school students.

HIGHER HEALTH FOCUS AREAS



COVID-19

Disease prevention and control



GENDER-BASED VIOLENCE

Policy development, awareness, early detection and access to care



MENTAL HEALTH

Awareness, combating stigma, access to services



HIV, TB AND SEXUALLY TRANSMITTED INFECTIONS (STIs)

Prevention, early detection, access to treatment and support systems



SEXUAL AND REPRODUCTIVE HEALTH (SRH)

Contraception, antenatal care, prevention and early detection of certain cancers



LGBTQI+ HEALTH

Awareness, promotion of rights, sensitised health services



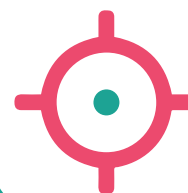
HARMFUL USE OF ALCOHOL AND DRUGS

Awareness, combating stigma, linkage to services



DISABILITY

Awareness, promotion of rights, access to services



HIGHER HEALTH GOALS

- **ENHANCE STUDENT HEALTH AND WELLNESS** in order to increase the proportion of students who complete their studies and to reduce the time it takes for students to graduate.
- **BUILD THE KNOWLEDGE, UNDERSTANDING, SKILLS AND CAPACITY** of students in relation to general health and holistic wellness.
- **HELP PRODUCE GRADUATES** who are well-prepared for family and career responsibilities by developing their grasp of the importance of health in these contexts.
- **DEVELOP A CAMPUS ENVIRONMENT** that is conducive to health by nurturing a commitment to human rights as a fundamental aspect of student development and wellbeing.
- **IMPROVING ORGANISATIONAL EFFECTIVENESS AND EFFICIENCY** in order to reduce organisational costs and increase funding for health and wellness-related programmes.
- **STRENGTHEN AND EXPAND PARTNERSHIPS WITH HIGHER EDUCATION INSTITUTIONS**, relevant government departments, funding and donor agencies, and other organisations active in our field.
- **STRENGTHEN AND DIVERSIFY OUR FUNDING BASE**, ensuring a variety of funding sources in order to secure operational sustainability.

STRATEGIC APPROACH

HIGHER HEALTH places a strong emphasis on promoting health and wellbeing and addressing various risks to health before harm occurs. For a young student population, we feel this is an obvious priority that promises greater benefits than a narrow focus on treating disease.

However, preventive and promotive health programmes are complex because the causes of ill-health are diverse. Besides the immediate biological factors, psychological pressures, lifestyle, and the social and physical environment all play a part. The health response needs to employ methods that address the range of causal factors.

Accordingly, HIGHER HEALTH:

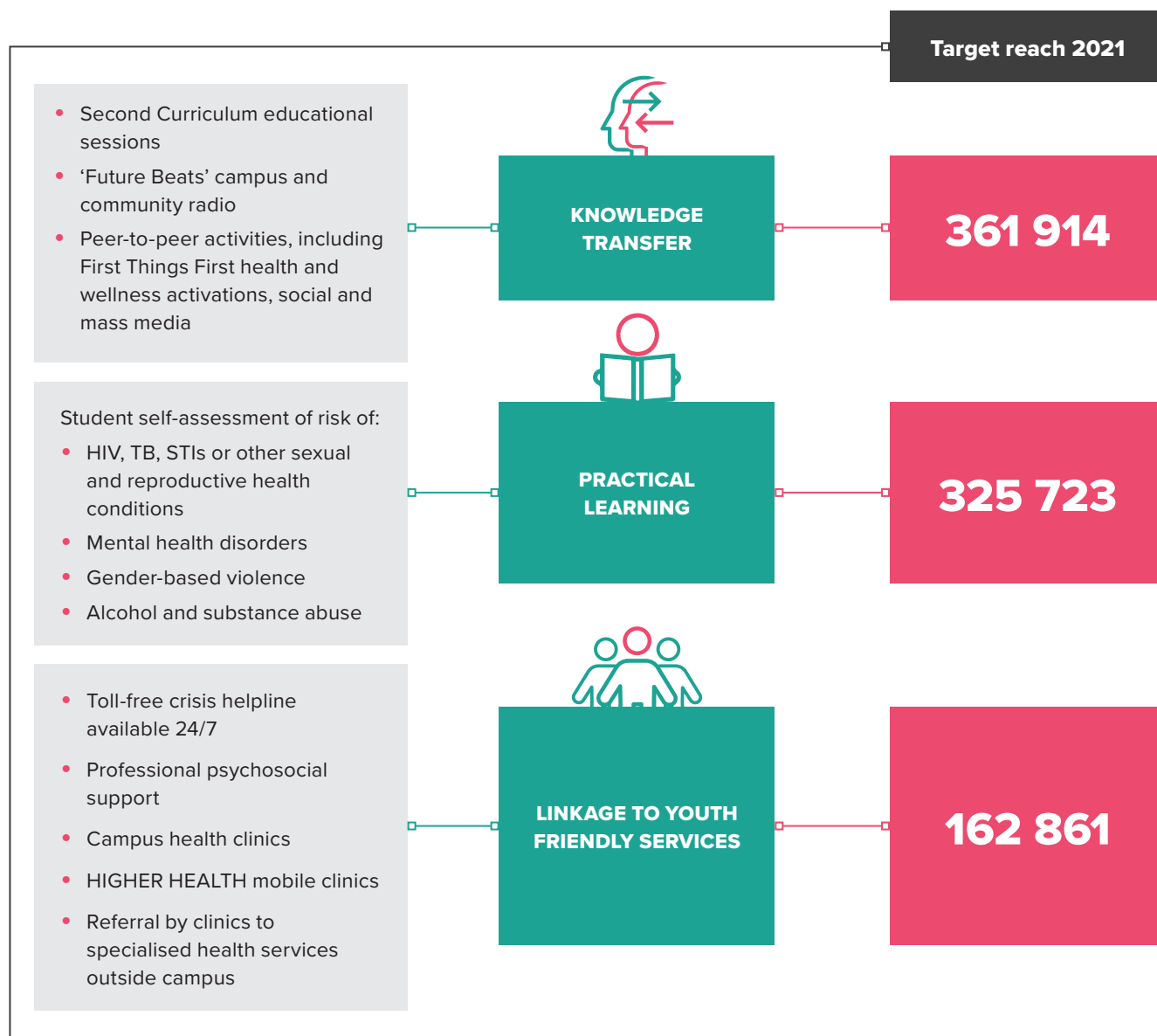
- Invests strongly in **health education**. Knowledge is empowering and, although it may not be sufficient in all cases to trigger a health-seeking response, it lays the foundation for further intervention.
- Mobilises the power of **peer influence**. Motivation and self-confidence are critical to an individual switching unhealthy habits for a healthier lifestyle and there is good evidence that peer educators – people like ourselves in terms of age and life experience – can unlock this change.

- Challenges students to reflect on their **health risks**. Risk perception is also an important factor in motivating behaviour change for healthier outcomes.
- Offers easy-to-reach solutions and support to reduce risk or mitigate any harm already done. These include **screening and testing** detect various diseases, provision of various **health products** (such as condoms and contraceptives) to ensure protection, and easy on-campus access to **youth-friendly health services**.
- Strives to build a campus **environment that nurtures health**. HIGHER HEALTH develops national policies and guidelines applicable to all campuses, spearheads interventions to combat stigma and discrimination that undermine the health of some students, and encourages open debate on sensitive health-related issues.

This thinking is encapsulated in the HIGHER HEALTH Model presented below. Some unique terms used in the model – such as First Things First activations and the Future Beats radio initiative – are explained later in this report. The model unfolds in an environment defined by the policies, strategies and guidelines HIGHER HEALTH has developed in collaboration with institutions.

OUR PURPOSE, STRATEGIC APPROACH AND SYSTEMS CONTINUED

Figure 3: HIGHER HEALTH Model for promotion and protection of student health



UNIVERSITY AND COLLEGE PARTNERSHIPS

Partnerships with public universities, public TVET colleges, and CET colleges are at the heart of HIGHER HEALTH's delivery system. The division of responsibility between our organisation and the institutions differs by sub-sector.

- In the university sub-sector, HIGHER HEALTH mainly works indirectly by providing policy and operational guidance, training for university staff, access to health supplies, and modest grants for implementation of the HIGHER HEALTH Model. Universities tend to have resources – for example, campus health clinics, student counselling services, transformation programmes, and campus radio stations – that are well-suited and willing to put the HIGHER HEALTH programme into practice.
- In the TVET sub-sector, colleges vary in terms of size and resources. But very few have adequate resources to implement the HIGHER HEALTH Model without assistance. We therefore augment their

capacity by assigning at least one campus health and wellness coordinator (CHWC) to each college, driving partnerships with provincial and district health services for the operation of clinics on campus, and providing mobile clinics in more remote areas as well as mental health services.

- In the CET sub-sector, our collaboration is still evolving as this sub-sector was first included in HIGHER HEALTH programming in 2020. Presently the focus is on policy and operational guidance, training of staff, and provision of essential health supplies. CHWCs provide support to CET colleges and some of the larger CET centres.

In all, HIGHER HEALTH caters to a sector with some two million students and employees. (See Figure 4.) Universities currently account for more than half the total, but government's strategy is to grow TVET colleges and CET colleges faster than universities over the next decade. The student population is much more dispersed in the TVET and CET sub-sectors, posing practical problems for health programme delivery.

Figure 4: Overview of the PSET sectors

	Public universities	TVET colleges	CET colleges
Institutions	26	50	9
Campuses/centres	108	274	3 276
Students	1 074 912	673 790	171 409
Staff	64 124	18 235*	13 236
HIGHER HEALTH coverage	100% of campuses	100% of campuses	100% of colleges 5% of centres

Source: Statistics on post-school education and training in South Africa, 2019. DHET, Pretoria: 2021

* Figure from TVET sub-sectors report for 2019/20 sector skills programme. ETD P SETA and DHET.

STUDENT VOLUNTEERS AS A DRIVING FORCE

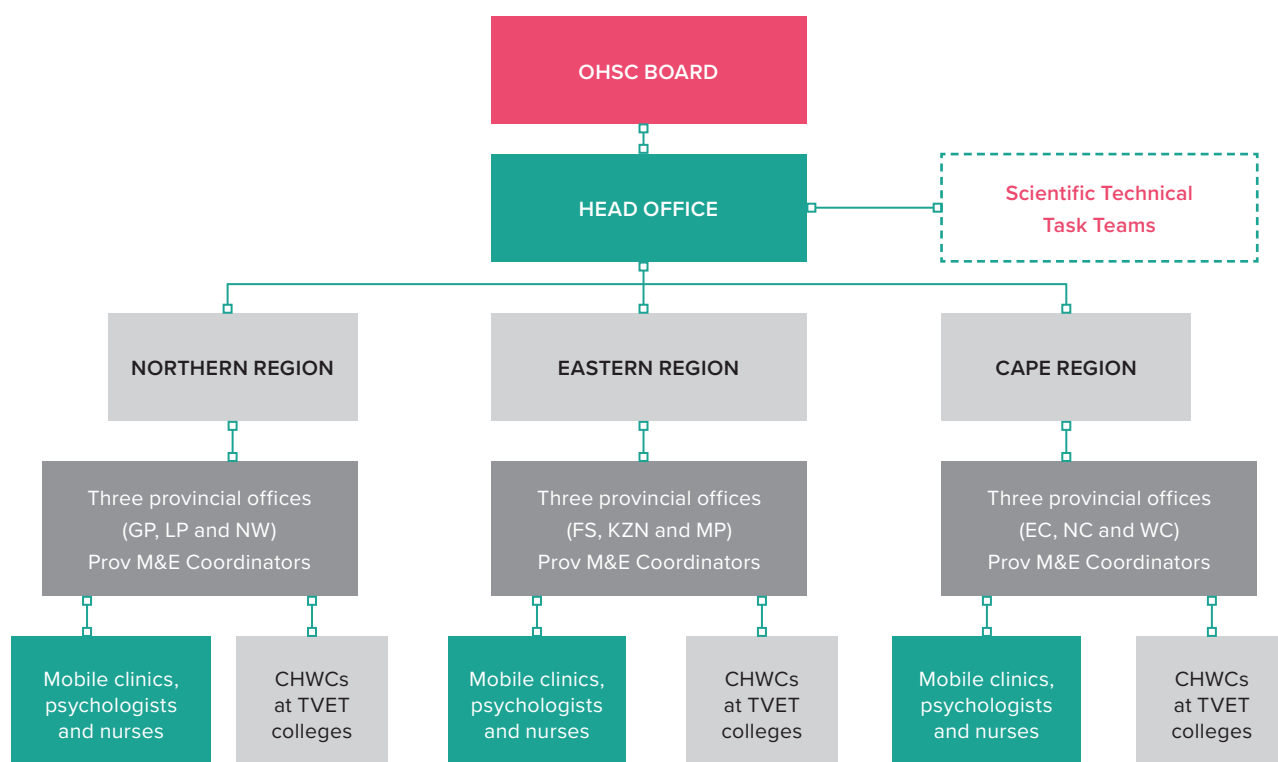
A valuable resource that all PSET institutions have is students and students drive the peer-to-peer engagement that lies at the heart of the HIGHER HEALTH Model. Each year a new cohort of peer educators is recruited, trained and deployed under the guidance of CHCWs.

In 2021, due to COVID-19, new students were not exposed to peer education during orientation week and recruitment drives for peer educators were adversely affected. Initiatives were taken to recruit peer educators through social media and digital platforms, but the approach was less effective than face-to-face recruitment. Despite this, 301 peer educators signed up at universities while at TVET colleges a total of 1 320 peer educators and peer mobilisers came forward and received relevant training. Some peer educators volunteered as COVID Warriors, and underwent training to implement COVID guidelines and protocols. They played an important role in screening individuals accessing campus.

STRUCTURES FOR MANAGEMENT AND DELIVERY

The past three years have seen a significant decentralisation of HIGHER HEALTH with the establishment of three regional and nine provincial offices. The most immediate reason for this was the provision of direct support to TVET colleges through dedicated CHWCs and, subsequently, mobile clinics and mental health services. The added benefit has been greater accessibility to all universities, TVET colleges and CET colleges around the country and more hands-on management of activities, including better monitoring. Although CHCWs are based at TVET colleges, they are accountable to HIGHER HEALTH provincial offices.

Figure 5: National structure of HIGHER HEALTH



HIGHER HEALTH has encouraged the establishment of dedicated management structures at institutions to oversee the health programme. For purposes of operational planning, implementation and ongoing service provision, we have forged working relationships with campus health services, student affairs offices, student representative councils, campus radio stations, managers of student residences, security managers, lecturers in key disciplines, and the peer education network which comprises mentors and students. The quality of implementation of the total HIGHER HEALTH Model depends on the contribution of these diverse elements of campus life. HIGHER HEALTH seeks to make their responsibilities more rewarding through high-quality capacity building and the provision of tools and resources that enhance performance.



03

Delivery of core programmes

DELIVERY OF CORE PROGRAMMES

In 2021, although the sense of crisis provoked by the COVID-19 pandemic had eased, supporting the PSET sector's response to COVID-19 remained HIGHER HEALTH's most pressing activity. The arrival in South Africa of adequate supplies of COVID-19 vaccines represented a great opportunity for the sector, while further waves of infection constituted a serious threat.

Our coordinated sector-wide response enabled universities and colleges to return to more extensive classroom-based teaching and learning. This increased presence of students and staff on campuses also opened the door to normalisation of other health interventions that had been scaled back in 2020. The return to contact teaching was swifter in the TVET college sub-sector which showed remarkable resilience in restoring a range of youth health programmes on campus. The university sub-sector continued hybrid and online learning to a greater degree and this impacted on its ability to engage students in health activities.

During 2021/22 HIGHER HEALTH also began to offer more systematic support to CET colleges and centres, although much developmental work remains to be done in this sub-sector.

ENABLING THE SECTOR'S COVID-19 RESPONSE

HIGHER HEALTH's role in relation to COVID-19 has been to provide guidance and develop resources to enable institutions in the PSET sector to play their role in curtailing the spread of infection while continuing their core educational and research functions. In the course of 2021/22, HIGHER HEALTH:

- Accessed expert advice to provide the sector with policies and guidelines that are evidence-formed.
- Continued to build the capacity of institutional management, staff and student volunteers to plan and implement infection control measures and other COVID-19 services.
- Coordinated a sector-wide vaccination strategy to provide both staff and students convenient access to vaccination as soon as they were eligible.
- Provided tools for use across the sector – such as the digital screening app, HealthCheck; mass communication via campus and community media; and a toll-free crisis helpline, which originally assisted with the psychosocial stresses of COVID-19 but has since taken on a broader significance in student life.

Policy development

Vaccination against COVID-19 – particularly mandatory vaccination as a condition of accessing public spaces – has been a hotly debated issue worldwide. Although scientific evidence has unequivocally demonstrated the massive benefits of vaccination, these facts are often swamped by preconceptions, emotion and the deliberate spread of false “information”.

South Africans are as divided as any other country and throughout the year HIGHER HEALTH took every opportunity to advocate that people opt for COVID-19 vaccination on the grounds of its proven safety and efficacy, particularly in reducing severe illness and the risk of death.

The Minister of Higher Education, Science and Technology requested HIGHER HEALTH to prepare the first draft of a vaccination policy for the sector. We were mindful of the sensitivity of the issue and the long-established autonomy of universities, some of which introduced their own vaccine mandates by the end of 2021. Accordingly, we conducted extensive discussions with various stakeholders, including university senate members, deans of students, student organisations SAUS and SATVETSA, and campus managers. These engagements and the input of various experts informed the draft policy, which HIGHER HEALTH submitted to the DHET in February 2022 for further discussion and development.

Another critical area for new guidance as campuses filled up was the physical distancing of students in classrooms, lecture halls, libraries, other learning spaces and residences.

Coordinating the PSET vaccination programme

During the first half of 2021, HIGHER HEALTH coordinated the development of a vaccination strategy for the PSET sector. This guided the provision of infrastructure to administer vaccines to all students and staff. It was developed to dovetail with the national vaccination programme, utilising community sites where appropriate and setting up fully fledged vaccination sites on campus where the volume of students and staff demanded this and vaccination points on other campuses. Details are set out in Table 2.

The strategy dealt with the nuts and bolts of the sector's demand for vaccines and the supply via public and private health systems. This required the closest communication both with institutions and service providers. The strategy also set out principles for institutions embarking on vaccination which included bringing key stakeholders on board and conducting a robust fact-based communication campaign on vaccination for the entire campus community.

Table 2: The vaccination service network

Type of service	Institutions served
Accredited campus vaccination site with vaccine storage	19 public universities
Campus vaccination points	7 public universities
	50 TVET colleges (200 campuses)
Link to closest community or workplace vaccination site – total of 4 000 sites	9 CET colleges (1 800 centres)
	137 private universities and colleges
	21 SETAs Trades-related institutions

THE VACCINATION PROCESS

Vaccination of the PSET community proceeded in phases. Healthcare workers on the staff of universities and colleges and students studying in health faculties were first, followed by staff classified as essential workers, then staff according to age category and, finally, the broad student body.

THE VACCINATION RESPONSE

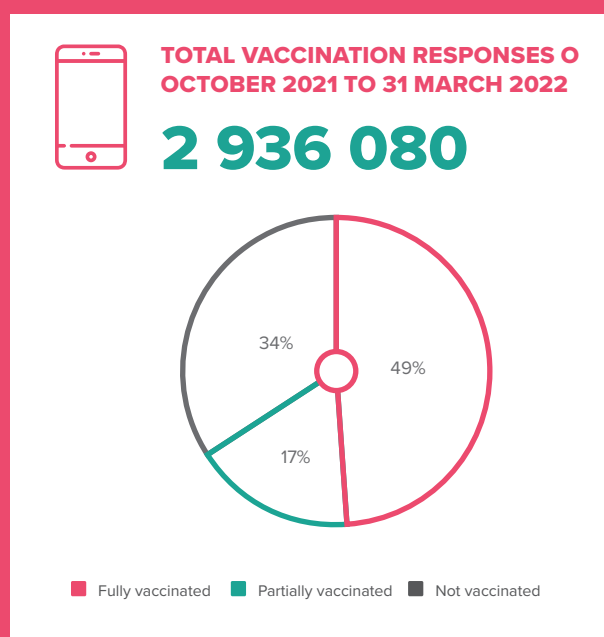
There is no accurate measure of the sector-wide vaccine response. However, the HealthCheck screening app (see below) was enhanced during the year to allow users to record their vaccination status. A total of 2 936 080 vaccine responses were recorded on HealthCheck between October 2021 and 31 March 2022. About 49% of users indicated they were fully vaccinated and 17% partially.

VERIFYING VACCINATION STATUS

At the start of the 2022 academic year institutions that had introduced vaccine mandates faced the challenge of verifying the vaccination status of students as they arrived on campus to register. The sheer volume of students would make manual verification unworkable.

HIGHER HEALTH engaged with the Department of Health which adjusted the Electronic Vaccination Data System (EVDS) so that institutions could access official vaccination records directly. This enabled them to scan the QR code on the certificate presented by the student to establish its authenticity. HIGHER HEALTH developed a manual to assist institutions to use of this method of verification.

Figure 6: Reports of vaccination status on HealthCheck



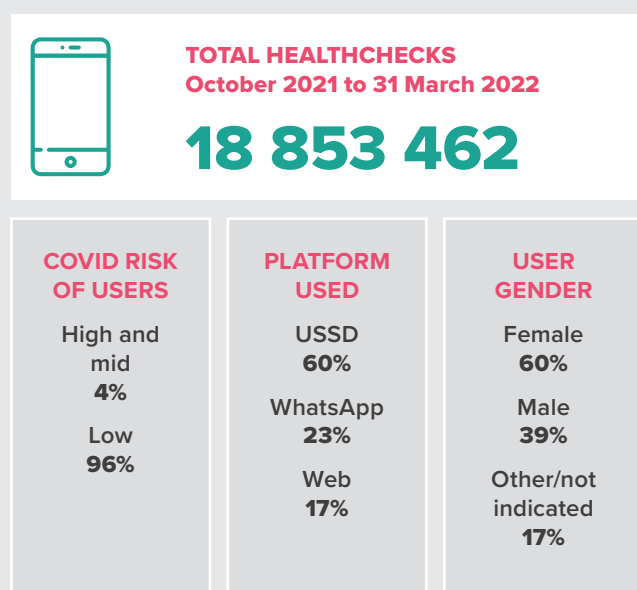
Maintaining prevention measures

Most students and staff only had access to vaccination in the latter part of 2021. Few had been vaccinated when the third wave, dominated by the severe delta variant of COVID, struck during the winter months. The sector's ability to sustain infection-control measures was therefore paramount.

The HealthCheck digital screening tool continued to be widely used during the year for students and staff to check their COVID-19 risk status before proceeding to campus, obtaining a daily health "passport" and passing speedily through the physical screening points at entrances to campuses.

Between its launch in 2020 and 31 March 2022, HealthCheck had been used on 18.85 million occasions. The ratio of high – or medium-risk results to total screenings was just under 4% – a seemingly low figure but possibly enough spread the virus to a considerable number.

Figure 7: Use of HealthCheck screening app



Building capacity for infection control

The protocols and guidelines that HIGHER HEALTH developed in 2020 to assist the sector to prevent the spread of COVID-19 continued to be applied during 2021. However, with periodic changes in government regulations, HIGHER HEALTH regularly checked whether guidelines were consistent with new regulations and advised institutions of any changes in implementation.

These regulatory changes, the inevitable turnover among students and staff, and the inclusion of the CET sector created continued demand for capacity development. The extent of this is indicated in the table that follows.

Table 3: COVID-19 capacity development in PSET sector Jan to Dec 2021

Category of personnel and volunteers	Number trained
Management and academic staff of institutions	21 635
Screening volunteers and peer educators	21 336
Frontline staff	2 780
All categories at universities and TVET and CET colleges	45 751

SEXUAL AND REPRODUCTIVE HEALTH PROGRAMME INCLUDING HIV AND TB

Sexual and reproductive health (SRH) features significantly in most comprehensive youth health strategies, including HIGHER HEALTH's. We strive to offer a full range of SRH services but the scale of delivery remains weighted towards the prevention of HIV. This is appropriate considering that South Africa still records some 200 000 new HIV infections a year and that the most vulnerable age for acquiring HIV is the late teens – which coincides with the first few years of PSET study.

New HIV infections have been declining in South Africa for almost two decades and our challenge is to speed up the process and focus on sections of the population that are most at risk of infection. Experts concur that the reduction in incidence of HIV was largely due to people with HIV reducing their viral loads to extremely low levels through ART. This is known as treatment as prevention (TASP) and also as "undetectable is untransmissible" (U=U). Additional reasons advanced for the decline in new infections are higher rates of condom use and medical male circumcision (MMC).

HIGHER HEALTH employs all three of the above prevention methods:

- We promote and provide HIV testing and link those who test positive with ART services.
- We advocate for condom use and distribute condoms widely on campuses.
- We educate on the benefits of MMC and make referrals to facilities that perform the procedure.

While TB is not sexually transmitted, it is tucked into the SRH programme because of its impact on HIV. It is not only the most life-threatening comorbidity for people living with HIV (PLHIV), but having TB greatly increases an individual's risk of acquiring HIV, and vice versa.

Prevention, early detection and treatment of other STIs, contraceptive counselling and provision, referral for antenatal care or termination of pregnancy, and screening for breast, cervical and prostate cancer round out the SRH service package.

The three-tier HIGHER HEALTH Model – knowledge transfer, practical learning, and linkage to services – firmly underpins the SRH programme. Psychosocial and environmental factors play a major role in exposure to HIV and other STIs and unplanned pregnancies. Our programmes to address GBV, harmful use of alcohol and drugs, mental health issues and difficulties experienced by LGBTQI+ students are also linked to more positive SRH outcomes.

Another fundamental feature of the SRH programme is advocating for sexual and reproductive health rights (SRHR) and embodying respect for these rights in campus clinic services. In society more broadly, young people are often judged and belittled by healthcare workers when they seek SRH services. HIGHER HEALTH strives, through its work with clinic staff, to establish a respectful environment for young people to meet their SRH needs.

SRH Programme methods

Health promotion

HIGHER HEALTH has developed a model of health promotion that combines:

- Knowledge transfer through the structured learning afforded by virtual and face-to-face learning sessions that form part of the Second Curriculum.
- A whole range of extramural peer-to-peer educational activities that not only impart information but bring a strong motivational energy to the programme.
- The Future Beats campus and community radio programme that facilitates dialogue on the social and cultural issues that impact on healthy lifestyle choices.
- Access to self-administered risk screening tools – usually used in the supportive environment of a Second Curriculum session or peer engagement – to assist students appreciate their personal level of health risk and encourage them to take constructive action.

Health services

Campus health services for HIV, TB and STIs cover both prevention and treatment and curative and include:

- HIV testing and screening for TB and STIs (and often, also, for non-communicable diseases such as diabetes and hypertension). These are available either at First Things First health days and activations or on-campus or mobile primary care clinics.

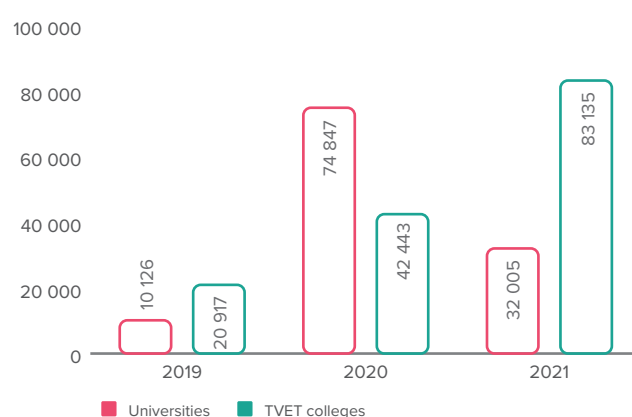
- Follow-up counselling after HIV testing with provision of ARV treatment where this is needed or referral if the campus facility does not offer ART.
- TB sputum collection or referral to a clinic for formal TB investigation when screening indicates the need for this.
- TB treatment or referral for TB treatment and related psychosocial support.
- Contraceptive counselling and supply of contraceptives, either at an on-campus clinic or by referral, pregnancy testing, and referral for antenatal care or termination of pregnancy.

Outputs and outcomes

The expectation for 2021 was that we would see a revival in uptake of campus-based health programmes which could not be easily accessed in 2020. This held true for TVET colleges where face-to-face teaching resumed to a significant degree. TVET colleges made impressive gains in the number of health and wellness days organised, students participating in the Second Curriculum, completing health risk self-assessments, and undergoing testing and screening.

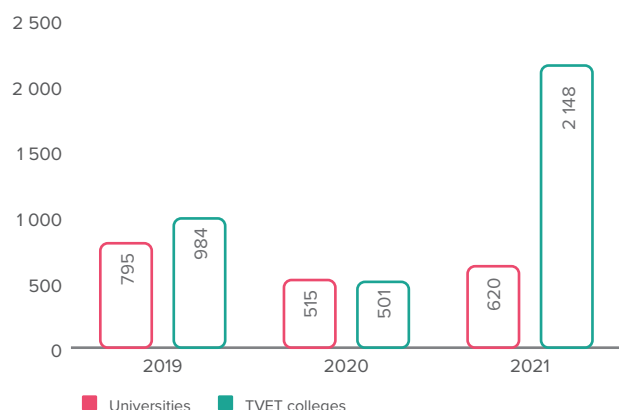
However, universities adhered largely to a hybrid model of learning and teaching for 2021. The patterns of programme participation in the sub-sector are difficult to understand. Second Curriculum participation dropped sharply although sessions can be held online, while risk assessments improved from a zero base in 2020.

Figure 8: Student participation in Second Curriculum sessions on HIV, TB and SRH



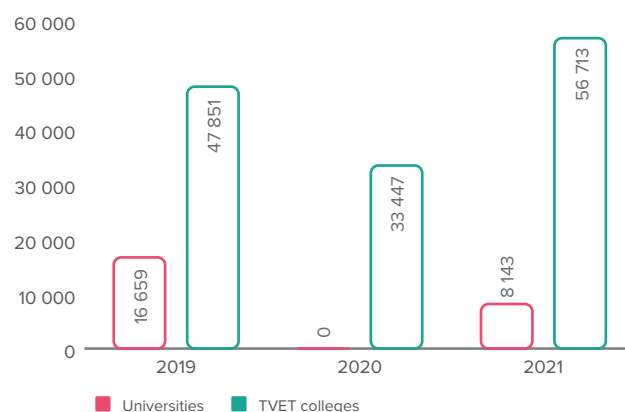
DELIVERY OF CORE PROGRAMMES CONTINUED

Figure 9: Health and wellness days held



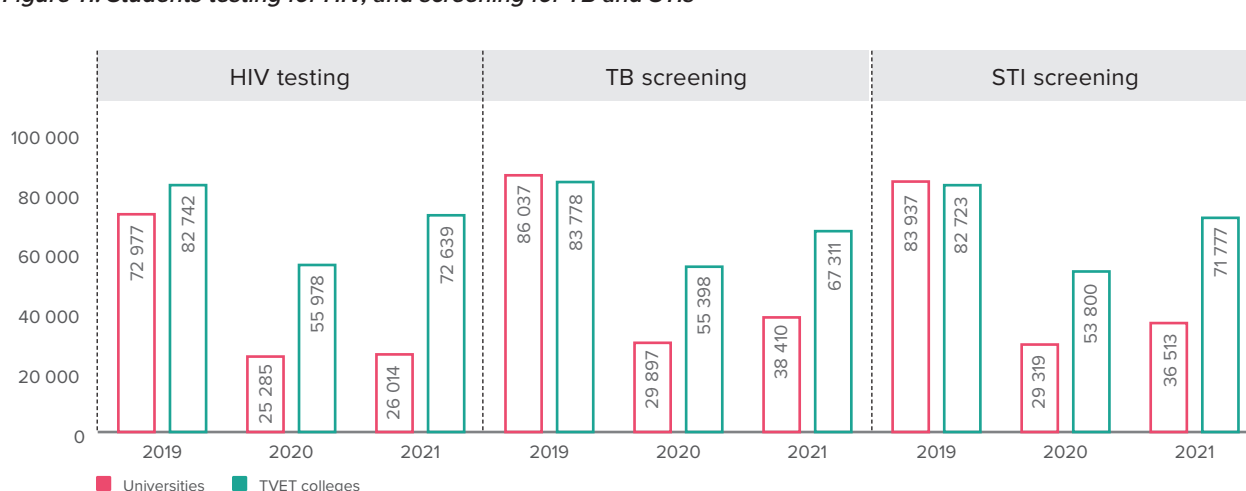
Overall, uptake of the HIV/STI/TB self-screening tool was significantly stronger in 2021 than in the previous year. Once again TVET colleges accounted for the bulk of growth. Nearly 16% of students performing self-screening identified themselves as at high risk of acquiring an STI or becoming pregnant unintentionally, and about 6% of self-screenerers were referred for care.

Figure 10: Students performing HIV, TB and SRH risk assessment



There were year-on-year gains at both TVET colleges and universities in the number of students testing/screening for HIV, TB and STIs. However, the relatively small recovery in HIV testing of university students was a matter of concern.

Figure 11: Students testing for HIV, and screening for TB and STIs



Through fixed or mobile campus clinics, or referral to community health facilities, HIGHER HEALTH facilitates access to the full toolbox of HIV prevention options as well as other SRH services. There was a promising improvement in access to certain products and services during the year, as reflected in Figure 12.

Our condom distribution in 2021 picked up modestly: a total of 6.3 million male and female condoms were distributed. HIGHER HEALTH supplies a relatively high ratio of female condoms to male condoms when compared to most other organisations. However, demand from vending machines installed as part of our Adolescent Girls and Young Women Programme (described later in this report) indicated that woman students generally prefer male condoms.

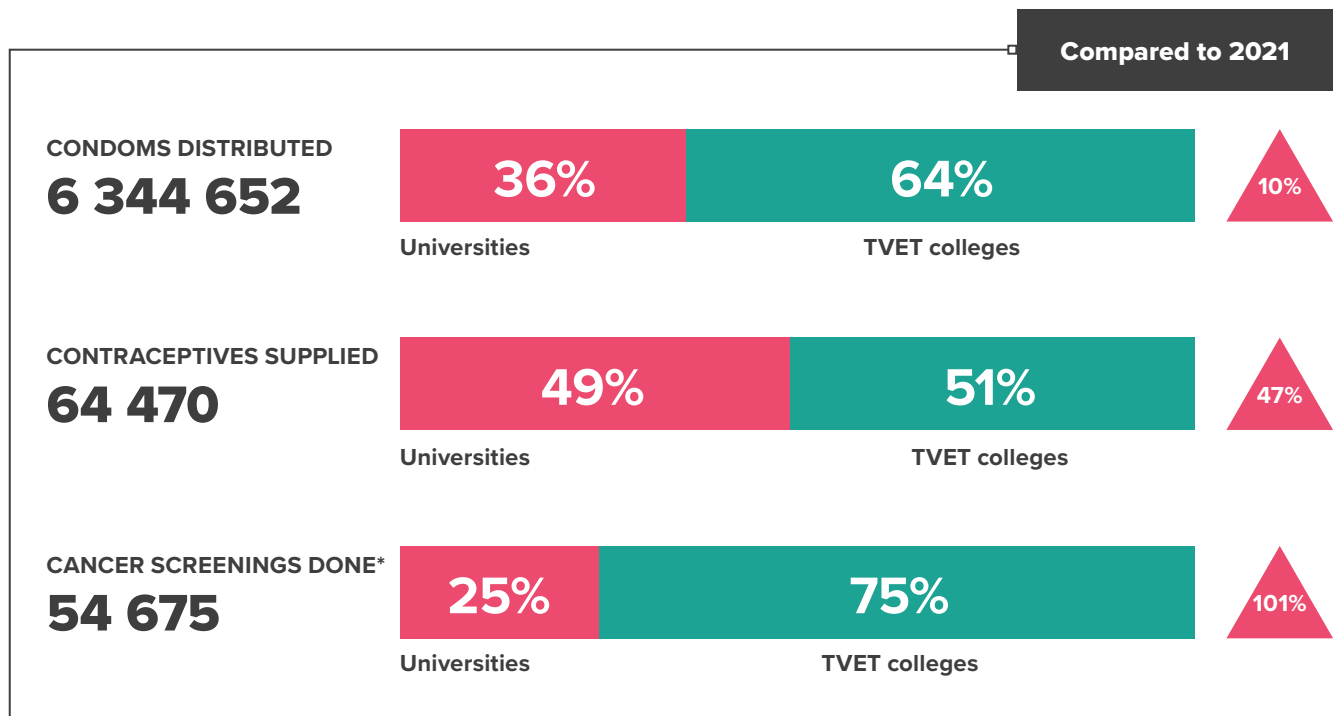
HIGHER HEALTH's promotion of pre-exposure prophylaxis (PrEP) in 2020 was hamstrung by conditions resulting from the COVID-19 pandemic. However, in 2021 interest in PrEP (which involves taking a daily ARV pill to prevent HIV acquisition) grew and 16 409 students received PrEP on campus or were referred to a clinic providing it.

There was modest demand (from 630 students) for medical MMC. Given the age of PSET students and the fact that circumcision is usually performed during adolescence, we do not expect huge demand.

A total of 1 399 students received ART at campus clinics or were referred to facilities in the community for treatment.

The uptake of contraception and requests for pregnancy testing suggest that our efforts to prevent unintended pregnancy among students require concerted attention. A total of 64 470 contraceptive products were supplied (either on campus or through referral) and 14 661 pregnancy tests were performed. While many students may opt for condoms as their preferred method of contraception, there is clearly space to promote long-acting contraceptive methods more positively. Campus health facilities referred 49 students for antenatal care and 393 for termination of pregnancy – and these numbers are certainly not the full picture.

Figure 12: Supply of SRH products and services 2021



*Screening covered breast, cervical and prostate cancers

SCREENING FOR NCDs

Presently HIGHER HEALTH does not have a full programme on non-communicable diseases (NCDs) but we encourage students attending our wellness days to request screening for diabetes, hypertension and high cholesterol. In 2021 a total of 149 760 screenings were performed, a considerably higher number than in the previous year.

MENTAL HEALTH PROGRAMME

Various studies have substantiated a need for mental health services for students at PSET institutions and HIGHER HEALTH's programme responds to this evidence. Depression, suicidal thoughts and anxiety are among the disorders that have commonly been identified among students. HIGHER HEALTH strives to make mental healthcare more accessible – and acceptable – to students by leveraging existing services at universities and providing dedicated professional care in the TVET college sub-sector.

Mental health is not only related to the unique stresses of transitioning to university life and the academic demands made on students. But there is a complex relationship between mental health and other health conditions and issues confronting a large number of students, including HIV, gender identity, sexual orientation, GBV, disability, the

use of alcohol and other substances, and stigma related to all the foregoing. HIGHER HEALTH's holistic approach to health and wellbeing recognises these linkages and attempts to address them.

The **objectives** of the HIGHER HEALTH mental health programme are to:

- Promote mental health.
- Prevent mental health disorders by addressing determinants as far as possible.
- Identify mental health problems early and provide timely assistance in a cost-effective, accessible, acceptable and affordable manner.

Methods of programme delivery

A brief description of how HIGHER HEALTH and PSET institutions have joined hands to promote mental health and consolidate care systems is presented below.

Table 4: Levels of psychosocial intervention and delivery systems

Level of psychosocial support	Delivery systems and structures
LEVEL 1	
• Creating awareness of mental health • Building resilience and self-help capacity • Anti-stigma activities • Psychoeducation • Self-screening on mental health risk • Suicide prevention • Modifying structural and environmental factors that affect mental health • Utilisation of Future Beats radio programme • Second Curriculum sessions on mental health	• Peer educator-driven dialogues and awareness campaigns, including distribution of IEC materials • Use of standardised self-assessment tool to identify students at risk • Developing guidelines, protocols and interventions to address risk factors in the campus environment
LEVEL 2	
• Access to crisis counselling and support	• Toll free helpline run jointly by HIGHER HEALTH and SADAG. This is available 24/7 on 365 days of the year and offer crisis support in several languages
LEVEL 3	
• Professional care by mental health professionals within the PSET sector • Referral to private mental health professionals • Referral to hospitals offering mental healthcare	• Clinical psychologists in HIGHER HEALTH provincial officers • Clinical psychologists and other counsellors at some institutions • Private sector mental health professionals, especially where students have medical aid • State and private hospitals, general/specialised

Figure 13: Enrolment in Second Curriculum module on mental health and GBV

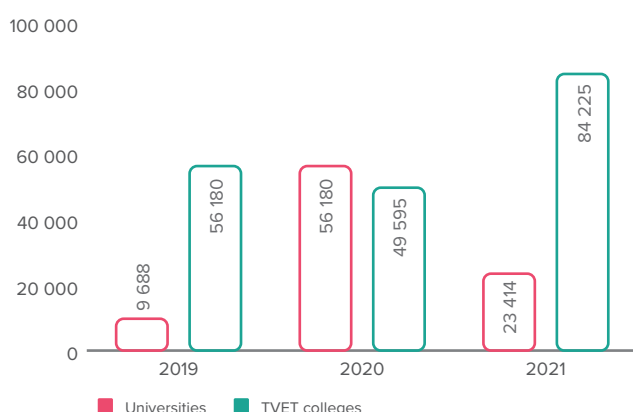
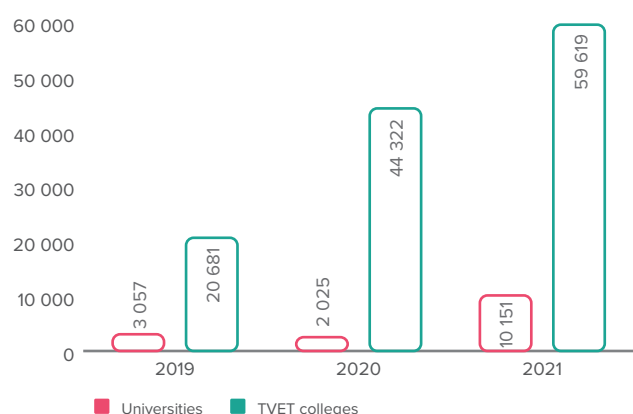


Figure 14: Completion of mental health and GBV risk self-assessment



Mental health and GBV comprise a single module of the Second Curriculum but, when it comes to identifying students at risk, a separate analysis is performed for GBV and mental health. In 2021, about 13% of students who completed the self-assessment were at high risk of mental health disorders.

The toll-free crisis helpline

The toll-free crisis helpline completed its first full year of operation in 2021 and registered 11 297 calls and 908 SMS requests for assistance. The average number of calls a month was 941 which is somewhat higher than the average monthly figure of 855 for the last five months of 2020.

Helpline staffers are trained to deal with the immediate crisis and link the caller to further help, where necessary. They have crisis counselling skills and knowledge of available mental health resources, which enables them to facilitate access to the right kind of care – be it admission to hospital, accessing community care (including medication) at a clinic, or linking the caller to a relevant support group.

Table 5: Reasons for students contacting hotline or seeing HIGHER HEALTH psychologists

Top four reasons 2021		Top four reasons 2020	
Relationship issues	28%	Academic stress and anxiety	30%
Depression	15%	General stress and substance abuse	22%
Anxiety	12%	Abuse and trauma	19%
Academic stress	11%	Depression and suicidal ideation	18%

Users of the helpline and psychology services showed a different pattern of need in 2021 to 2020. In the first year of the COVID-19 pandemic stress related to academic performance was pervasive. This moderated in 2021, as face-to-face learning expanded and the online learning experience became more familiar. Depression has remained a major factor in students' lives, and a more generalised form of anxiety was prevalent in 2021. The striking feature of the last year was the emergence of relationship issues as the main cause of psychosocial crises.

Dedicated psychologist services

HIGHER HEALTH has a total of eight qualified psychologists on its staff and they have made a significant contribution to meeting the psychosocial needs of TVET college students, especially outside of the main urban areas. Our psychologists are based at our provincial offices in Gauteng, Eastern Cape, KwaZulu-Natal and Mpumalanga.

The HIGHER HEALTH service is time-bound, consisting of a maximum of six sessions. Many students' issues can be resolved within this limit and, where this is not the case, students are referred to private or public practitioners and facilities for extended or specialised care.

In 2021, HIGHER HEALTH's psychologists had a total of **6 221** sessions with students, translating to an average of **518** sessions a month.

While this is impressive, overall awareness of mental health and the availability of psychosocial services is low among students and stigma still deters some from seeking help. The task of advocating for mental health and raising awareness must be sustained.

On-campus psychosocial support

In 2021, a relatively large number of students – 25 964 – received psychosocial support from resources on campus, including student counselling services, primary healthcare clinics, and HIGHER HEALTH CHWCs based at TVET colleges. This support may extend to the provision of counselling but in many cases the individual would be referred to a more specialised service.

Wellcome Trust/SAGE Bionetwork research partnership

HIGHER HEALTH is participating in a study on the feasibility of developing a global databank on youth mental health by means of smartphone-based data collection. The study involves South Africa, India and the United Kingdom, is funded by the Wellcome Trust and managed by SAGE Bionetwork in Seattle. HIGHER HEALTH, Nelson Mandela University and Fort Hare University comprise the South African partners. A total of 912 South African students signed up to submit daily information over a period of three months and provide “passive data”, for example, on sleep patterns, exercise and cell-phone use. Data analysis commenced in the first quarter of 2022 and the study is expected to yield a number of journal articles.

GENDER-BASED VIOLENCE PROGRAMME

South African women have a 45.6% lifetime risk of experiencing GBV, a rate well above the global average of 35% (World Health Organization) and campuses are not exempt from this epidemic. For example, the SA Medical Research Council has found 10% of reported rapes occur in the PSET sector.

Features of campus life give rise to specific forms of GBV, such as sex-for-marks or other campus benefits, and spying or intruding on students in residences. While women bear the major brunt of GBV, gender non-conforming individuals are also frequently targeted.

Research conducted by HIGHER HEALTH in 2009, 2014 and 2016, found:

- Neither students nor staff members – service workers and academics – felt safe on campus.
- Both male and female students held beliefs that not only supported male dominance but might enable violence, for example:
 - Nearly three out of 10 took the view that a girl had no right to refuse sex with her boyfriend.
 - More than half felt forced sex with someone you knew did not amount to sexual violence.
 - Transactional sex is common on campus (in one study 14% of respondents said they engaged in it) and this blurs the boundaries in terms of consent and abuse.

It is clear that GBV is a present threat to students in the PSET sector but the majority of institutions are poorly equipped to take action to prevent it, assist survivors and institute appropriate action in relation to perpetrators. HIGHER HEALTH's role is to enable an appropriate set of responses across the PSET sector.

Our engagement with the issue of GBV began several years ago with awareness-raising and advocacy activities designed to win this “silent epidemic” a place on the sector's health agenda. Progress has been gradual.

Through policy and guideline development and capacity building, HIGHER HEALTH assists institutions implement the Gender-based Violence and Femicide National Strategic Plan, GBV policies of the DHET, and the GBV module of the Second Curriculum. Our latest protocols and guidelines on GBV, launched by Deputy Minister of Higher Education and Training, Mr Buti Manamela, in August 2021, were:

- Procedural guidelines on sexual and gender-related misconduct.
- Implementing protocols on rape and sexual assault.
- Implementation protocol on the code of ethics.

HIGHER HEALTH also supports:

- **Capacity development** of campus staff involved in providing support to survivors of GBV. These include protection services, clinical staff, student support structures, and student representative councils.
- **Health promotion and building social awareness**, using the broad platforms established by HIGHER HEALTH: the Second Curriculum, the peer educator network and the Future Beats radio initiative. There was a year-on-year increase in the number of students enrolled in the Second Curriculum sessions on GBV and mental health, with total participants numbering 107 639 (2020: 105 775). This growth was all in the TVET sub-sector.
- **Violence prevention**, using of the self-screening tool to identify vulnerable students assess their needs and provide assistance. TVET colleges accounted for 85% of student self-screenings on GBV and mental health, although the number of screenings at universities (10 151) was nearly five times the number completed in 2020 (2 025).
- Various services for victims, survivors and perpetrators, including:
 - A crisis resource, the HIGHER HEALTH 24-hour toll-free hotline.
 - Clinical care for physical injury and prevention of pregnancy, HIV and other STIs.
 - Professional psychosocial care offered by HIGHER HEALTH counsellors and psychologists.
 - Peer support through GBV support groups.

In 2021/22 HIGHER HEALTH drafted a raft of additional GBV protocols to be finalised in the year ahead. These included:

- A whistle-blowing protocol.
- Campus minimum standards protocol.
- Rape and sexual assault protocol.
- Femicide protocol.



Figure 15: GBV interventions in 2021

UNIVERSITIES	TOTAL NUMBER OF STUDENTS	TVET colleges
23 414	ENROLLED IN GBV SECOND CURRICULUM 107 639	84 225
10 151	PERFORMED GBV RISK ASSESSMENT 69 770	59 619
857*	REFERRED FOR GBV PSYCHOSOCIAL SUPPORT 3 795	2 938*
885	REPORTED GBV CASE ON OR OFF CAMPUS 1 321	436

* Risk screening is only one of several ways students are referred for psychosocial support and at universities it is a relatively minor route to care.

PROGRAMMES TO SUPPORT VULNERABLE POPULATIONS

Various health strategies identify sections of the population whose life circumstances or behaviour place them at increased risk of ill-health. Often these groups are socially marginalised, disempowered and stigmatised, and these factors further heighten their likelihood of experiencing poor health. Interventions designed to empower members of vulnerable populations and combat discrimination and stigmatisation are therefore at the heart of health programmes to benefit them.

HIGHER HEALTH has prioritised four sections of the student population as vulnerable groups:

- Woman students, who generally fall into the category of adolescent girls and young women (AGYW).
- The campus LGBTQI+ community.
- Students with disabilities.
- Students who use alcohol or drugs in a harmful way or are dependent.

Each vulnerable group requires a unique approach and HIGHER HEALTH's response is, regrettably, not equally advanced for all of them.

Programme for Adolescent Girls' and Young Women

In South Africa and much of sub-Saharan Africa AGYW are especially vulnerable to HIV infection. This association has led international funding agencies to prioritise AGYW and HIGHER HEALTH has had the benefit of a generous three-year grant from the Global Fund to Fight HIV, TB and Malaria to provide a comprehensive health programme for women students.

The AGYW Programme aims to reduce woman students' vulnerability to HIV infection, protect them from unwanted pregnancy, lower their risk of GBV, and increase their chances of completing their chosen course of study. It operates on 20 TVET college campuses located in 11 sub-districts, most of which are under-served.

HIV testing and core HIV prevention services are funded by the Global Fund and provided directly on campus as part of HIGHER HEALTH programming, while additional services may be available from campus clinics or through referral to partners in the community.

HIGHER HEALTH has employed peer group trainers (PGTs) at all campuses participating in the AGYW Programme. PGTs are responsible for:

- Enrolling young woman students in the programme.
- Peer education, demand creation for services, distribution of IEC materials and organising First Things First activities.

- Facilitating students' use of risk self-assessments and HIV testing facilities.
- Delivering a core package of services to each enrolled student.
- Ensuring linkage to services, including treatment, care and support.
- Facilitating access to SRH products.

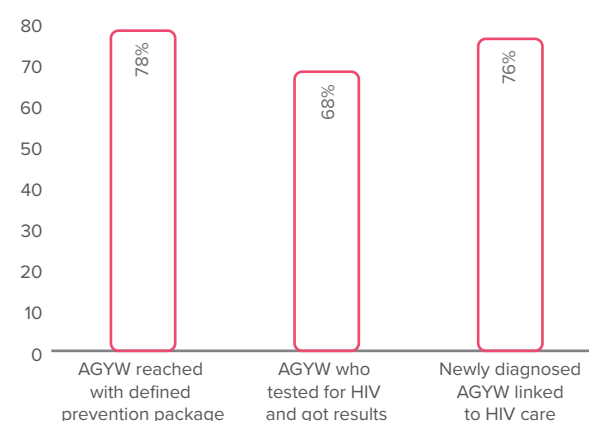
SRH products – such as condoms, sanitary towels and pregnancy tests – were supplied from August 2021 via 36 vending machines placed on participating campuses. Enrolled students were given pin codes to acquire products free of charge, in privacy. The response to the machines has been extremely positive. However, some campuses experienced start-up issues in terms of the computer hardware and internet connectivity required to support vending machines. These campuses resorted temporarily to manual supply systems while the issues were addressed.

The AGYW Programme is tracked in terms of meeting numerical targets in three key areas:

- Providing a package of HIV prevention services to AGYW at participating campuses.
- Facilitating HIV testing for AGYW and the receipt of their test results.
- Linking AGYW who test positive for HIV to treatment services and other care.

Performance against these targets is reflected in Figure 16.

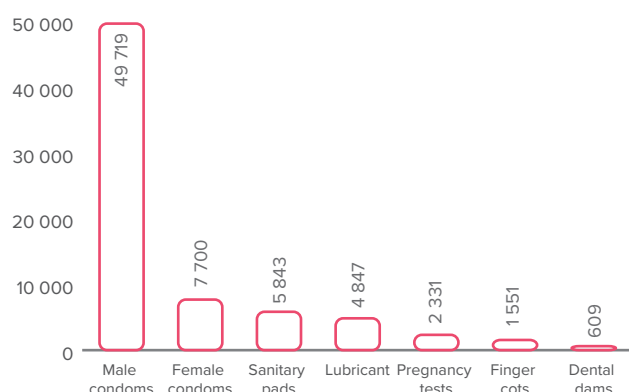
Figure 16: Performance against targets for AGYW Programme Jan – Dec 2021



The Global Fund expects a minimum performance of 75% of target. Following an implementation audit conducted by the Global Fund Office of the Inspector-General in October 2021, HIGHER HEALTH put in place measures to improve performance, focusing on sub-districts that were under-performing. While we were able to meet targets in terms of the reach of prevention services and linkage of newly diagnosed students to care, the HIV testing rate was still below target at project close-out.

Figure 17 shows the demand for SRH commodities from vending machines. This data on user preferences will inform the stocking of machines in the future. HIGHER HEALTH monitors demand for pregnancy tests and will increase SRH interventions at colleges where requests for pregnancy tests are particularly high. Male condoms were by far the most-requested commodity. Research may be needed to learn more about female condom use.

Figure 17: Uptake of SRH products at vending machines August 2021 – March 2022



Alcohol and Drug Abuse Prevention Programme

HIGHER HEALTH's own research has indicated that a substantial proportion of students and staff at universities and TVET colleges abuse alcohol:

- In our 2009 knowledge, attitudes and practices (KAP) study, 35% of university students reported having been drunk in the month prior to the survey.
- In our 2014 KAP study at TVET colleges, 60% of students and 58% of staff reported abusing alcohol regularly.

The World Health Organization estimates that seven in 10 men who consume alcohol in SA – young men and adults – engage in “heavy episodic drinking”. The association of immoderate alcohol use with crime and violence (including domestic violence and GBV), risky sexual behaviour, road traffic and other accidents, and mental and physical disorders is well documented.

Other drugs that are commonly used among students in the PSET sector are tobacco cigarettes, dagga, nyaope, whoonga and tik.

The **objectives** of HIGHER HEALTH's Alcohol and Drug Abuse Prevention (ADAP) Programme are:

- Reducing the overall level of alcohol use.
- Delaying the consumption of alcohol and use of specified drugs.
- Reducing the harmful use of alcohol and drugs.
- Minimising the harm associated with use of alcohol and drugs.

Knowledge building on harmful use of alcohol and drugs forms part of a Second Curriculum module that also deals with disability, sexual orientation and gender non-conformity. Self-screening for alcohol and drug abuse risk is being phased in. In 2021, only Gauteng and KwaZulu-Natal implemented it on a meaningful scale. Gauteng facilitated 3 465 self-screenings and identified 46 students with an addiction, nearly all of whom were referred for help. While KwaZulu-Natal facilitated 1 102 self-screenings, only four were considered to have an addiction problem.

Table 6: Self-screening for risk of alcohol and drug abuse and follow-up assistance 2021

Students performing self-screening	Students referred for professional assessment	Students diagnosed with alcohol/drug addiction & assisted
5 037	189	70

Supporting students with disabilities and LGBTQI+ students

HIGHER HEALTH has adopted a three-pronged approach to the health and wellbeing of students with disabilities and LGBTQI+ students.

- Sensitising and educating the broader PSET community about people with disabilities and the LGBTQI+ population. Stigma thrives on disinformation and education is a significant weapon against it. The Second Curriculum and peer-led activities serve this objective.
- Including members of these populations in health initiatives and empowering them to take action in their own interests. Peer-facilitated support groups form part of this approach.
- Equipping staff of health services to respond to the specific health needs of students with disabilities and LGBTQI+ students. Lack of healthcare provider knowledge about their unique needs and how to respond to them constitute a barrier to care.

The use of the Second Curriculum in support of vulnerable population programmes is still quite limited. The number of participants in 2021 totalled 44 773 - about half the number attending other Second Curriculum modules.

UNIVERSITY PROGRAMME

HIGHER HEALTH plays an enabling role in respect of the country's 26 public universities, helping to equip them to implement the HIGHER HEALTH Model. The following are our key interventions: support.

- Capacity development and skills training.
- Strategic and technical guidance on student health, wellness and development.
- Sectoral coordination and stakeholder engagement.

Grant management and support

HIGHER HEALTH offers direct grants to the student affairs departments of all public universities to support their health and wellness activities.

April 2021 was the start of the new grant period for 2021 - 2023 and grants ranging from R300 000 to R500 000 per university were approved for 23 of the 26 public universities. In addition, in recognition of the constraints imposed by COVID-19, it was agreed that unspent grant allocations from the previous two-year cycle could be carried over and used during 2021. The first disbursement, amounting to 25% of the new grant amount and totalling R2.65 million, was made in August 2021.

Table 7: University grants approved for April 2021 – March 2023

University	Grant amount
Cape Peninsula University of Technology	R500 000
Central University of Technology	R500 000
Durban University of Technology	R500 000
Mangosuthu University of Technology	R400 000
Nelson Mandela University	R500 000
North West University	R500 000
Rhodes University	R400 000
Sol Plaatje University	R300 000
Stellenbosch University	R500 000
Tshwane University of Technology	R500 000
University of Cape Town	R450 000
University of Fort Hare	R500 000
University of the Free State	R500 000
University of Johannesburg	R500 000
University of KwaZulu-Natal	R500 000
University of Limpopo	R380 000
University of Mpumalanga	R350 000
University of Venda	R400 000
University of Zululand	R480 000
University of Pretoria	R500 000
University of the Western Cape	R500 000
University of the Witwatersrand	R500 000
Vaal University of Technology	R500 000
Total	R10 660 000

A critical deliverable in terms university grant agreements is student use of HIGHER HEALTH's risk self-assessment tools. Universities have been somewhat slow to incorporate this aspect of our model, largely because many students studied online in 2020 and 2021 and universities had difficulties finding appropriate electronic platforms for remote completion of the self-assessment tool. Various options were tested and the 2021 academic year showed some progress in this area.

Table 8: University performance against targets in first year of grant award

Deliverable	Percent of target achieved in first year
Enrolment in Second Curriculum educational sessions (HIV/TB/STIs and mental health/GBV)	63%
Self-screening questionnaires completed (HIV/TB/STIs and mental health/GBV)	21%
Students linked to treatment, care and psychosocial services	111%

Capacity development and skills training

HIGHER HEALTH provides training to various categories of university students and staff to enable them to implement sectoral health policies and guidelines and deliver our programmes.

- In October 2021, a total of 198 peer education coordinators from universities nationwide participated in "train the trainer" workshops on delivery of the Second Curriculum, the use of risk-screening tools and facilitation of peer-to-peer sessions. The aim was to enable them to train others at their institutions to implement these aspects of our programme.
- In November, 119 students recently elected to student representative councils attended workshops on the HIGHER HEALTH Model and key student health programmes.

Strategic and technical guidance

HIGHER HEALTH provides ongoing guidance on student health, wellness and development to various university role-players. Interventions during this reporting period included:

- Engagements with university COVID-19 task teams and student leadership to provide guidance on vaccination policies for institutions.
- Collaboration with the EVDS technical team to rollout the user manual for COVID-19 vaccine certificate verification by universities.
- Representations to the Ministerial Advisory Committee on COVID-19 on reducing the physical distancing required by regulation.



Sectoral coordination and stakeholder engagement

As a national entity with an established track-record, HIGHER HEALTH is well placed to provide a platform for engagement among stakeholders within the higher education health and wellness space. During the year we facilitated discussions with provincial health departments and partners such as the Marie Stopes Foundation, Clinton Health Access Initiative (CHAI), Youth Against AIDS, Wits Reproductive Health and HIV Institute (Wits RHI) and UNESCO in order to expand campus health services and improve access to healthcare consumables.

COMMUNITY EDUCATION COLLEGE PROGRAMME

HIGHER HEALTH first extended support to the community education and training (CET) sub-sector in 2020, at the height of the first COVID-19 wave. This crisis-driven initiative opened the door to a more purposeful partnership in 2021 when HIGHER HEALTH received substantial grants from the HWSETA and the DHET to establish health programmes for CET colleges.

The programme will be built on the model tested and refined in the TVET college and university sub-sectors. However, it will be adjusted – as needed – for the CET environment. The sub-sector comprises nine provincial colleges and thousands of delivery centres across the country, which often utilise school infrastructure after hours.

The CET Programme aims to:

- Expand the development of skills among students and staff.
- Strengthen CET colleges' systems and structures for providing primary healthcare (PHC) services.
- Respond to the COVID-19 pandemic and maintain safe teaching and learning environments across the CET sub-sector.

More specific objectives are to:

- Build the capacity of students and staff at 200 large CET centres to establish and maintain systems for a safe learning and teaching environment in the presence of COVID-19.
- Develop management skills and health infrastructure to implement the COVID-19 guidelines.
- Enable CET colleges to provide routine PHC interventions through the mobilisation of student volunteers and the deployment of HIGHER HEALTH staff at identified centres.
- Roll out a dynamic social and behaviour change campaign that focuses on the intersection between human behaviour and health, and utilises health promotion and preventive health interventions. Special attention will be paid to COVID-19, mental health, GBV, HIV, TB and STIs, and SRH.

Programme methods

The capacity development methods that will be used in the CET sub-sector will include – but not necessarily be limited to – the following:

- Standardised training to fill generalised knowledge and skills gaps in respect of selected health issues among CET college partners.
- Tailored training to address an institution's specific needs and challenges, and equip its human resources with relevant knowledge and skills.
- Mentoring and coaching, through periodic visits to colleges and centres and the use of technical specialists to teach and support college staff.
- Communities of practice, meeting physically and virtually, for relevant implementers and stakeholders to discuss challenges and share resources and lessons learned.

Table 9: Outputs in CET sub-sector Aug 2021 to Feb 2022

Performance indicator	Number
COVID-19 capacity building: managers and staff	4 299
COVID-19 guideline implementation training: various staff categories	6 692
COVID 19 capacity building: student volunteers and peer educators	1 078
Screening stations established	348
CET centres involved in HIGHER HEALTH implementation	157
CET students linked to HIGHER HEALTH care and support programmes	1 148

The outputs reflect a strong drive to build capacity for delivery – especially in relation to COVID-19 – and the beginnings of implementation in accordance with the HIGHER HEALTH model.

COMMUNICATION PROGRAMME

Communication is not only a way of informing people about HIGHER HEALTH's work but it is also a force that drives many of its programmes. Use of mass communication channels expands and reinforces the health promotion and health education activities undertaken on campuses. It also enables HIGHER HEALTH to participate in public debates on youth health issues and advocate for the kinds of action we understand to be imperative.

During the reporting period, HIGHER HEALTH combined various communication channels to address the public and key stakeholders: students and staff of universities and colleges, funders and programme partners, and role-players in the health and PSET environment.

Mass media communications

HIGHER HEALTH uses its presence in print, broadcast and digital media to increase the visibility of our organisation, inform audiences of our campus health programmes and present our position on important public health and social issues affecting young people.

Between January and December 2021, HIGHER HEALTH received strong media attention:

- Coverage in print, digital and broadcast media reached a cumulative audience of 546 634 646 people. Many individuals would have encountered multiple reports and are counted for each exposure.
- The value of coverage totalled over R21 million when calculated at advertising value equivalent (AVE) rates.

Key factors in HIGHER HEALTH's ability to leverage free mass media coverage are the Chief Executive Officer's willingness to work with journalists and his grasp of the issues in fast-developing news stories.

HIGHER HEALTH seldom purchases advertising airtime, but in May 2021 we developed public service announcements on COVID-19 in the PSET sector and flighted these on commercial and campus radio stations. They featured Deputy Minister Manamela, Dr Ahluwalia and some students.

Social media communication

HIGHER HEALTH has a dynamic social media presence on Twitter, Facebook and LinkedIn. Educational and motivational postings occur daily on these platforms, which are also used to advertise events and webinars. We currently have 846 followers on LinkedIn, 8 778 on Facebook and 2 288 on Twitter.

A mini-campaign to commemorate World AIDS Day 2021 was boosted and performed well.

Monitoring indicated there were more than **16 000** users, **22 000** impressions and **7 300** engagements.

Information, communication and education (IEC) materials

In 2021 HIGHER HEALTH developed and distributed nearly 30 000 posters and a million pamphlets dealing with COVID-19 and other key focus areas. Similar quantities were disseminated early in 2022 as students returned to campus. Posters were displayed in high-traffic areas of campus and repeatedly reminded students about healthy choices and the resources they can use. Corporate materials, such as banners, table cloths, foam hands and pledge walls, were procured to enhance the visibility of HIGHER HEALTH activities.

Table 10: Quantities of educational and branding materials distributed in 2021

	Pamphlets			Posters	Branding items		
	HIV, TB & SRH	GBV & mental health	COVID-19		T-shirts	Pledge walls	Foam hands
TVET colleges	183 000	183 000	183 000	20 000	13 500	200	1 230
Universities	134 000	134 000	134 000	10 000	8 000	143	1 010
Total	317 000	317 000	317 000	30 000	21 500	343	2 240

HIGHER HEALTH developed and distributed an induction video aimed mainly at first year students. This provides information on the student health and wellness programme and encourages students to participate. It can be viewed on YouTube: <https://www.youtube.com/watch?v=6j79l6F3F2w&feature=youtu.be>

HIGHER HEALTH guides the content of broadcasts throughout the year by circulating a schedule of themes and topics. In this way we ensure Future Beats supports all our focus areas and carries through our holistic approach. Podcasts of the broadcasts are available on the stations' social media.

Future Beats campus and community radio programme

One of HIGHER HEALTH's most consistent interventions is the Future Beats programme which harnesses the credibility and creativity of campus and community radio stations to produce and air youth-friendly health and wellness content.

Future Beats marked its eighth birthday in 2021/22 and proved its continued relevance by partnering, once more, with 20 campus and community radio stations. These gave HIGHER HEALTH access to a combined listenership of more than 1.1 million on campus or in neighbouring communities. This audience was reached on a weekly basis with new health-themed material.

Each of the stations received a grant from HIGHER HEALTH to produce and air youth-friendly health and wellness content. There were approximately 100 radio journalists across these stations and, via a series of webinars, HIGHER HEALTH developed their capacity to undertake health reporting.

In 2021, every station broadcast two shows a week on COVID-19. HIGHER HEALTH made a grant to the Voice of Wits (VOW) to develop these shows and syndicate them to other stations. VOW won the content producer award in the campus and community radio category at the 2021 South African Radio Awards for its COVID Report show.

All stations are also assisted to produce and air content every week on other student health, wellness and development topics.



DELIVERY OF CORE PROGRAMMES CONTINUED

Table 11: Campus and community radio stations participating in 2021

Station	Community or campus	Province	Listenership	Social media reach	Grant amount
Unitra FM	Community	EC	240 000	341	R30 000
Madibaz FM	Campus	EC	15 000	15 752	R20 000
Forte FM	Campus	EC	7 000	7 107	R20 000
VOW FM	Campus	GP	20 000	39 279	R60 000
Rhodes Music Radio	Campus	EC	21 838	21 838	R20 000
Alfred Nzo	Community	EC	138 000	6 796	R30 000
CUT FM	Campus	FS	38 000	49 114	R25 000
Kovsie FM	Campus	FS	28 003	9 683	R25 000
Barberton Radio	Community	MP	55 000	12 383	R30 000
SMU FM	Campus	GP	5 000	6 500	R20 000
Valley FM	Community	WC	121 000	15 000	R30 000
Univen Radio	Campus	LP	9 000	15 570	R20 000
Radio Turf	Campus	LP	6 000	14 000	R20 000
UCT Radio	Campus	WC	1 000	16 899	R20 000
Mdantsane FM	Community	EC	61 000	3 900	R30 000
Mahikeng FM	Community	NW	88 000	33 232	R30 000
Maputaland Radio	Community	KZN	85 000	6 913	R40 000
TUT FM	Campus	GP	54 000	7 500	R30 000
Tshwane FM	Campus	GP	38 000	27512	R30 000
Emalahleni FM	Community	MP	97 000	20796	R30 000
Total			1 127 841	239 367	R560 000

Thina Abantu Abasha (DREAMS TAA)

Johnson & Johnson funds a radio intervention for AGYW called Thina Abantu Abasha and in 2021, for the fourth year in a row, it selected Future Beats as an implementer. The project involved the broadcast of 14 packaged mini-dramas on selected campus and community radio stations: VOW FM and Tshwane FM in Gauteng, Alfred Nzo in the Eastern Cape, and Maputaland Radio in KwaZulu-Natal. Collectively these stations have 331 500 listeners. Thina Abantu Abasha deals with women's health, women's empowerment and GBV, poverty and youth unemployment, and social justice more broadly.

Digital dialogues

HIGHER HEALTH used digital meeting platforms fruitfully during the course of the reporting period.

We organised a limited number of **public webinars with high-profile speakers**, designed to attract large audiences. Such events were extremely successful and included:

- 18 May 2021: Lecture by Prof Salim Abdool Karim on long-term systems for continuing teaching and learning during pandemics. About 1 160 participants.
- 27 August 2021: Webinar to mark the release of tools for implementation of the PSET GBV Policy Framework. More than 800 participants.

- 19 November: Webinar anticipating the fourth wave of COVID-19 and exploring its likely impact on the 2022 academic year. About 850 participants.
- 16 March 2022: HIGHER HEALTH recognition awards ceremony, a hybrid event anchored in Tshwane with Minister Nzimande delivering the key note address. Large audience – SABC live broadcast.

We also coordinated a larger number of **targeted digital briefings and consultations** with key stakeholder groups. Many of these related to the planning of the sector's vaccination strategy and the drafting of its policy on vaccine mandates. Attendance at these meetings ranged from a dozen to 200, depending on the stakeholder category.

Digital events organised by our partners – for example, universities, government departments and USAf – and a range of other organisations, gave HIGHER HEALTH many opportunities to reach wider audiences. Dr Ahluwalia was in demand as a speaker at these events, which included national and international conferences conducted on digital platforms.

Visible oversight and leadership support

Throughout 2021/22 the HIGHER HEALTH programme enjoyed an extraordinary level of interest and engagement from Minister Nzimande and Deputy Minister Manamela.

They visited a number of university vaccination sites to confirm that they were operating smoothly and lend support to the vaccination initiative by taking their jabs on campus. The year saw a cautious return to face-to-face campus events attended variously by the Minister, Deputy Minister and European Union ambassador.

Table 12: High-level events between January and December 2021

Date	Event	Institution	Comments
13 Apr	Monitoring of district development model	University of Zululand, TVETS and CETs in Zululand District	Participation by Minister Nzimande
21 Apr	Signing of European Union grant agreement	Not applicable	The CEO and EU Ambassador Dr Riina Kionka signed a three-year partnership agreement
22 Apr	GBV and mental health dialogue	Mthashana TVET College, Vryheid	Addressed by Minister Nzimande and panel of students. On-site audience of 150. Live TV broadcast
24 Apr	Visit of support following campus fire	University of Cape Town	The CEO accompanied Deputy Minister Manamela to provide material and medical support
18 May	Ministerial monitoring of vaccination roll out	Edendale Hospital, a teaching facility of UKZN	Minister Nzimande conducted visit and had his first jab
21 May	Launch of COVID Warriors	Tshwane South TVET College	Deputy Minister Manamela recognised the work of peer educators called COVID Warriors
27 May	Certification ceremony for COVID Warriors	University of Fort Hare	EU Ambassador Kionka presented certificates of recognition to peer educators
14 Jun	Release of results of HIGHER HEALTH/HSRC COVID-19 impact study	Cape Peninsula University of Technology	The release was attend by the Minister and doubled as a vaccination site oversight visit
1 Jul	Ministerial monitoring of vaccine roll out	University of Pretoria	Minister Nzimande viewed the site and had his second jab
10 Aug	Monitoring of vaccination roll out	Tshwane University of Technology	Conducted by Deputy Minister Manamela who had his first jab there
4 Oct	Monitoring of vaccination roll out	Esayidi TVET College	Deputy Minister Manamela visited campus as part of government's Vooma Vaccination Campaign
26 Nov	Vaccination roll out monitoring and First Things First activation	Thekwini TVET College	Deputy Minister Manamela visited as part of government's Vooma Vaccination Campaign

CEO's stakeholder engagements

Of equal importance to HIGHER HEALTH's public events are the behind-the-scenes interactions of the CEO and his management team with key partners and stakeholders. These are too numerous to mention individually but fall into the following categories.

- Participation in national government technical structures on prevention of HIV, GBV, COVID-19, National Health Insurance (NHI) and mental health.
- Exploration of partnerships with international funding agencies and UN structures.
- Engagement with role-players in skills development for the health sector on creating and funding accredited training courses.
- Meetings with trade unions in the PSET sector and human resources managers of institutions.
- Discussions with representative bodies of university and TVET college students.



04

Human resources

HUMAN RESOURCES

The staff complement of HIGHER HEALTH remained stable in 2021/22 after rapid expansion in the previous two years. Our human resources management capacity at head office was strengthened in recognition that the recent growth in our regional and provincial teams had created a need for stronger support.

At the end of the 2021/22 financial year, HIGHER HEALTH had 124 employees, fractionally fewer than a year earlier. Our employees are broadly representative of the general population, with members of previously disadvantaged groups constituting the great majority of staff members. In terms of gender, HIGHER HEALTH's team is strongly skewed in favour of women.

CHWCs constitute the largest single category of HIGHER HEALTH employees. Many of them have an undergraduate degree or diploma but not a specific professional qualification. The majority are young and HIGHER HEALTH has often afforded them entry into the job market. Nurses and psychologists working within our provincial offices and professionals at head office constitute the next largest category of personnel.

Figure 18: HIGHER HEALTH employees by occupational category

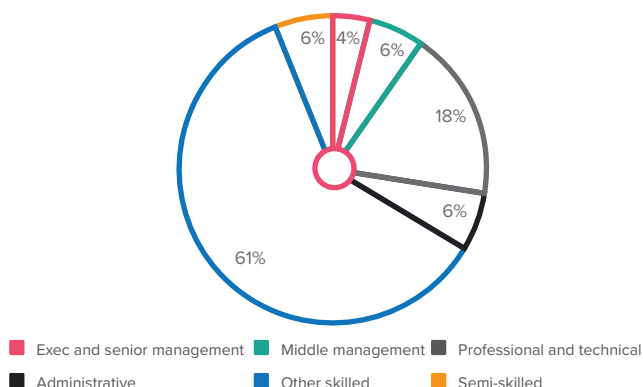
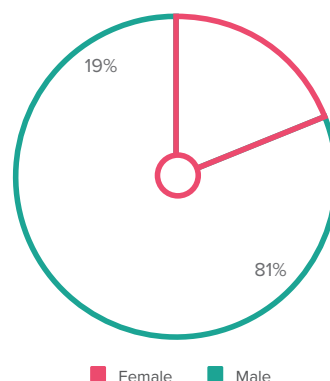


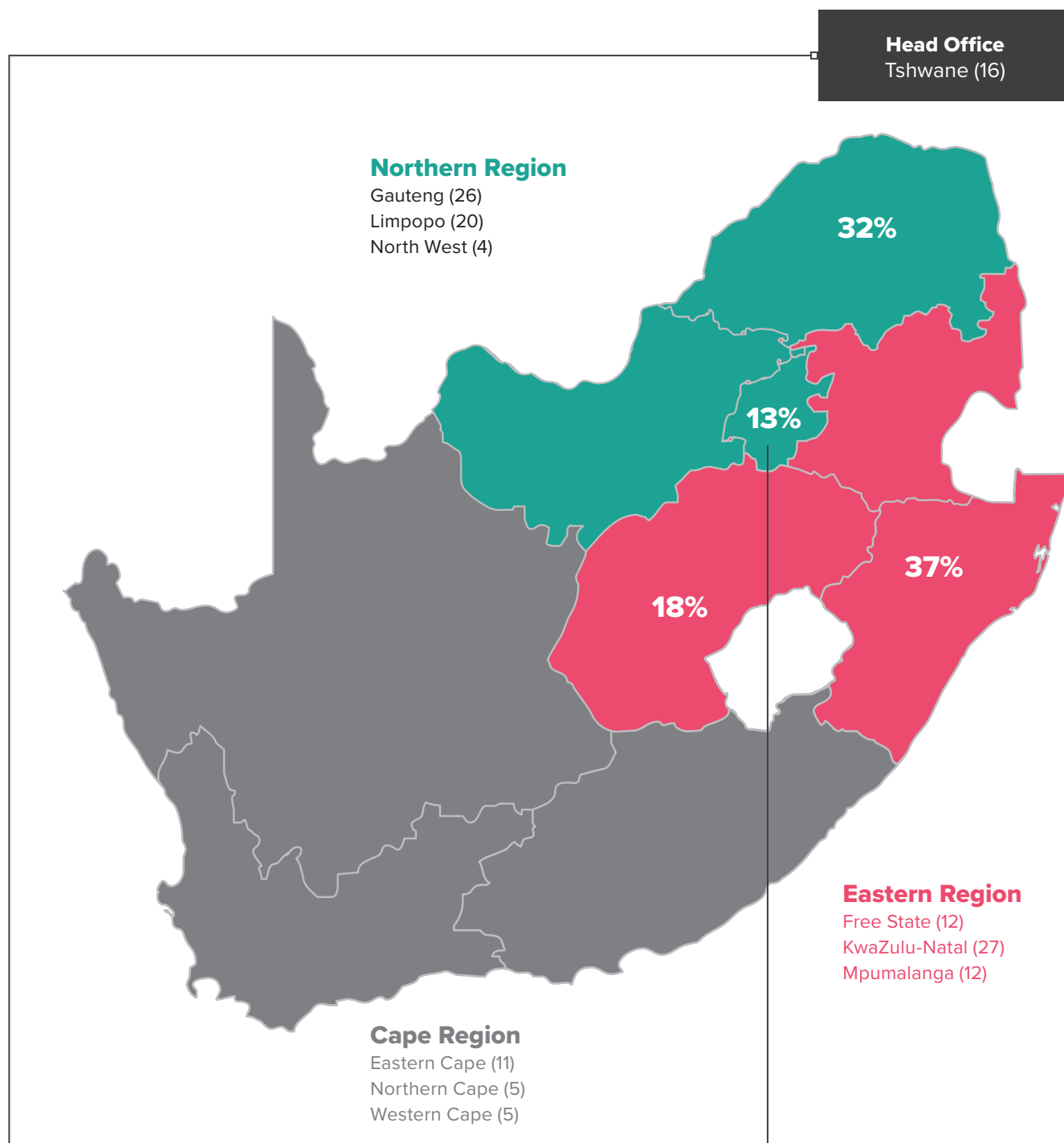
Figure 19: HIGHER HEALTH employees by population group



A national presence

HIGHER HEALTH has an office in every province, with CHWCs dispersed across TVET colleges. Our Gauteng and KwaZulu-Natal offices are actually larger than our head office. The distribution of employees is shown in Figure 20.

Figure 20: HIGHER HEALTH's national footprint: percentage of staff by region and number per province



With decentralisation, HR policies and systems become increasingly important to ensure consistency across offices. In the past year, a standard performance framework was developed. Similarly, the need arose to address matters of organisational culture and values more formally, and HIGHER HEALTH launched such an initiative for its staff early in 2022.

A skills audit was undertaken to inform the plan for building organisational capability. Recruitment of additional staff was initiated towards the end of the financial year in anticipation of a new grant from the Global Fund.



05

Governance

GOVERNANCE

HIGHER HEALTH is a non-profit company, registered in terms of the Companies Act. The strategic direction of the organisation and oversight of its management and performance vest in a Board of Directors.

The Board has been constituted to include representation of stakeholders that have been key to HIGHER HEALTH's development (USAf, South African Public Colleges Organisation [SAPCO] and the DHET) and to ensure that the organisation has access to individuals with special expertise. The current chairperson is Prof Wim de Villiers, Vice-Chancellor of Stellenbosch University, and the Deputy Chairperson is Mr Sanele Mlotshwa, Rector of Majuba TVET College. A full list of members appears below.

In terms of HIGHER HEALTH's memorandum of incorporation, the Board is required to meet at least three times a year. During 2020/21, it met three times, once in a face-to-face session and twice using a digital meeting platform.

Table 13: Members of HIGHER HEALTH Board of Directors April 2021 to March 2022

Board member	Organisation represented/area of expertise
Prof Willem de Villiers – Chairperson Vice-Chancellor, Stellenbosch University	Medical science Management
Mr Sanele Mlotshwa – Deputy Chair President of SAPCO; Rector, Majuba TVET College	Representing 50 TVET colleges
Prof Ahmed Cassim Bawa CEO, USAf	Representing 26 universities
Mr Mahlubi Mabizela Chief Director: University Policy & Development Support, DHET	DHET University Branch
Mr Brian Madalane Principal, Northern Cape Urban TVET College; Secretary-General, SAPCO	Higher education and training Institutional management
Prof Thokozile Valerie Mayekiso Vice-Chancellor, University of Mpumalanga	Higher education and training Institutional management
Prof Nancy Refilwe Phaswana-Mafuya Professor of Epidemiology and Public Health, University of Johannesburg	Public health Social determinants of health
Mrs Ashwanthee Singh Acting Deputy Director-General, TVET Branch, DHET	DHET TVET Branch
Dr Doeke Tromp Former CFO, Tshwane University of Technology	Financial management Strategic planning
Ex officio Dr (Prof) Ramneek Ahluwalia CEO of HIGHER HEALTH	HIGHER HEALTH

The Board's Audit & Risk and Finance & Investment Committee, which has an independent chair, Mr Theo Madurai (CFO of Tshwane University of Technology), met twice during the year.

The Human Resources and Remuneration Committee, chaired by Prof Bawa, met twice in the reporting year.



06

Annual financial statements

This section comprises financial information, reports required by statute, and extracts from the annual financial statements for the year ending 31 March 2022.

GENERAL INFORMATION

Country of incorporation and domicile	South Africa
Nature of business and principal activities	Health and wellness services and capacity development in higher education institutions which include universities and TVET colleges
Directors	Dr D Tromp Mr B Madalane Prof W De Villiers Dr (Prof) R Ahluwalia Prof NM Phaswana-Mafuya Prof TV Mayekiso Mr M Mabizela Mrs A Singh Prof A Bawa Mr S Mlotshwa
Registered office	262 Rose Avenue Wild Olive Riverfalls Office Park Centurion 0132
Business address	262 Rose Avenue Wild Olive Riverfalls Office Park Centurion 0132
Postal address	262 Rose Avenue Wild Olive Riverfalls Office Park Centurion 0132
Bankers	ABSA Bank Limited Investec Bank Limited
Auditors	PricewaterhouseCoopers Inc Chartered Accountants (SA) Registered Auditors
Company registration number	2017/419731/08
Tax reference number	9864/591/16/0
Level of assurance	These annual financial statements have been audited in compliance with the applicable requirements of the Companies Act of South Africa.
Preparer	The annual financial statements were independently compiled by: Exponential Brilliance and Afrizen Accounting and Consulting (Pty) Ltd.

AUDIT COMMITTEE REPORT

We are pleased to present our report for the financial year ended 31 March 2022.

The Audit, Risk, Finance and Investment Committee is an independent statutory committee appointed by the directors of the company. The committee has conducted its affairs in compliance with its terms of reference.

1. Members of the Audit Committee

The Audit, Risk, Finance and Investment Committee is independent and consists of five independent, non-executive directors. It meets at least twice a year as per its terms of reference.

The chairman of the Board, chief executive officer, finance director, external auditor and other assurance providers (legal, compliance, risk, health and safety) attend meetings by invitation only.

The members of the Audit Committee are:

Name	Appointed	Institution
Mr Theo Madurai (Chairperson)	21/11/2017	Tshwane University of Technology – Chief Financial Officer
Prof TV Mayekiso	21/11/2017	University of Mpumalanga – Vice-Chancellor
Dr Doeke Tromp	21/11/2017	Independent
Mr Eminos Manyawi	15/3/2019	Sefako Makgatho Health Sciences University – Chief Financial Officer
Mr Brian Madalane	15/3/2019	Northern Cape TVET College – Principal

2. Meetings held by the Audit Committee

The Audit, Risk, Finance and Investment Committee performs the duties laid upon it by Section 94(7) of the Companies Act by holding meetings with the key role players on a regular basis and by the unrestricted access granted to the external auditor.

3. External Auditor

The Audit Committee has satisfied itself that the external auditor was independent of the company, as set out in Section 94(8) of the Companies Act 71 of 2008, which includes consideration of previous appointments of the auditor, the extent of other work undertaken by the auditor for the company, and compliance with criteria relating to independence or conflicts of interest, as prescribed by the Independent Regulatory Board for Auditors. Requisite assurance was sought and provided by the auditor that internal governance processes within the audit firm support and demonstrate its claim to independence.

The committee ensured that the appointment of the auditor complied with the Companies Act 71 of 2008, and any other legislation relating to the appointment of auditors.

The Audit Committee, in consultation with executive management, agreed to the engagement letter, terms, audit plan and budgeted audit fees for the 2022 year.

4. Annual financial statements

The Audit Committee has reviewed the accounting policies and the financial statements of the company and is satisfied that they are appropriate and comply with the International Financial Reporting Standards.

5. Effectiveness and internal control

The system of internal control applied by the entity over the financial reporting and risk management is effective, efficient and transparent. From the audit report on the financial statements and the management report of the external auditor, it was noted that no matters were reported that indicate any material deficiencies in the system of internal control or any deviations therefrom. Accordingly, the committee can report that the system of internal control over financial reporting for the period under review was efficient and effective.

On behalf of the audit committee



Theo Madurai
Chairperson of the Audit Risk Finance and Investment Committee
12 December 2022

DIRECTORS' RESPONSIBILITIES AND APPROVAL

The directors are required by the Companies Act 71 of 2008 to maintain adequate accounting records and are responsible for the content and integrity of the annual financial statements and related financial information included in this report. It is their responsibility to ensure that the annual financial statements satisfy financial reporting standards with regard to form and content and present fairly the statement of financial position, results of operations and business of the company, and explain the transactions and financial position of the business of the company at the end of the financial year.

The annual financial statements are based upon appropriate accounting policies, consistently applied throughout the company and supported by reasonable and prudent judgements and estimates. The directors acknowledge that they are ultimately responsible for the system of internal financial control established by the company and place considerable importance on maintaining a strong control environment. To enable the directors to meet these responsibilities, the Board of Directors sets standards for internal control aimed at reducing the risk of error or loss in a cost-effective manner. The standards include the proper delegation of responsibilities within a clearly defined framework, effective accounting procedures and adequate segregation of duties to ensure an acceptable level of risk. These controls are monitored throughout the company and all employees are required to maintain the highest ethical standards in ensuring the company's business is conducted in a manner that, in all reasonable circumstances, is above reproach.

The focus of risk management in the company is on identifying, assessing, managing and monitoring all known forms of risk across the company. While operating risk cannot be fully eliminated, the company endeavours to minimise it by ensuring that appropriate infrastructure, controls, systems and ethical behaviour are applied and managed within predetermined procedures and constraints.

The directors are of the opinion, based on the information and explanations given by management, that the system of internal control provides reasonable assurance that the financial records may be relied on for the preparation of the annual financial statements. However, any system of internal financial control can provide only reasonable, and not absolute, assurance against material misstatement or loss. The going-concern basis has been adopted in preparing the financial statements. Based on forecasts and available cash resources the directors have no reason to believe that the company will not be a going concern in the foreseeable future. The financial statements support the viability of the company.

The annual financial statements, which have been prepared on the going concern basis, were approved by the Board of Directors on 12 December 2022 and were signed on their behalf by:



Prof W de Villiers



Dr (Prof) R Ahluwalia

DIRECTORS' REPORT

The directors have pleasure in submitting their report on the annual financial statements of Higher Education Training Health Wellness Development Centre NPC for the year ended 31 March 2022.

1. Incorporation

The company was incorporated on 18 September 2017 and obtained its certificate to commence business on the same day.

2. Nature of business

Higher Education Training Health Wellness Development Centre NPC was incorporated in South Africa with interests in health and wellness services and capacity development in higher education institutions, which include universities and TVET colleges. There were no major changes herein during the year.

The operating results and statement of financial position of the company are fully set out in the attached financial statements and do not in our opinion require any further comment.

3. Board of Directors

The directors in office at the date of this report are as follows:

Board of Directors	Nationality
Dr D Tromp	South African
Mr B Madalane	South African
Prof W De Villiers	South African
Dr (Prof) R Ahluwalia	South African
Prof NM Phaswana-Mafuya	South African
Prof TV Mayekiso	South African
Mr M Mabizela	South African
Mrs A Singh	South African
Prof A Bawa	South African
Mr S Mlotshwa	South African

4. Directors' interests in contracts

During the financial year, no contracts were entered into which directors or officers of the company had an interest in and which significantly affected the business of the company.

5. Property, plant and equipment

There was no change in the nature of the property, plant and equipment of the company or in the policy regarding their use.

6. Events after the reporting period

All events subsequent to the date of the annual financial statements and for which the applicable financial reporting framework requires adjustment or disclosure have been adjusted or disclosed. The directors are not aware of any material event which occurred after the reporting date and up to the date of this report.

7. Going concern

The annual financial statements have been prepared on the basis of accounting policies applicable to a going concern. This basis presumes that funds will be available to finance future operations and that the realisation of assets and settlement of liabilities, contingent obligations and commitments will occur in the ordinary course of business.

DIRECTORS' REPORT CONTINUED

The directors believe that the company has adequate financial resources to continue in operation for the foreseeable future and accordingly the annual financial statements have been prepared on a going-concern basis.

The ability of the company to continue as a going concern is dependent on a number of factors. The most significant of these is that the directors continue to procure funding for the ongoing operations for the company where the need arises. Management is comfortable that Higher Health will continue to operate.

Further, Higher Health is actively involved in fighting against the pandemic and has been granted substantial funding to continue to do so. Higher Health is in the forefront of rolling out a vaccine strategy especially in the post-school education and training (PSET) sector.

8. Independent Auditors

PricewaterhouseCoopers Inc continued in office as auditors for the year under review.

9. Secretary

No secretary has been formally appointed during the current financial year.

Independent auditor's report

To the Directors of Higher Education Training Health Wellness Development Centre NPC

Our opinion

In our opinion, the financial statements present fairly, in all material respects, the financial position of Higher Education Training Health Wellness Development Centre NPC (the Company) as at 31 March 2022, and its financial performance and cash flows for the year then ended in accordance with International Financial Reporting Standards and the requirements of the Companies Act of South Africa.

What we have audited

Higher Education Training Health Wellness Development Centre NPC's financial statements set out on pages 11 to 31 comprise:

- the statement of financial position as at 31 March 2022;
- the statement of comprehensive income for the year then ended;
- the statement of changes in equity for the year then ended;
- the statement of cash flows for the year then ended; and
- the notes to the financial statements, which include a summary of significant accounting policies.

Basis for opinion

We conducted our audit in accordance with International Standards on Auditing (ISAs). Our responsibilities under those standards are further described in the *Auditor's responsibilities for the audit of the financial statements* section of our report.

We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our opinion.

Independence

We are independent of the Company in accordance with the Independent Regulatory Board for Auditors' *Code of Professional Conduct for Registered Auditors* (IRBA Code) and other independence requirements applicable to performing audits of financial statements in South Africa. We have fulfilled our other ethical responsibilities in accordance with the IRBA Code and in accordance with other ethical requirements applicable to performing audits in South Africa. The IRBA Code is consistent with the corresponding sections of the International Ethics Standards Board for Accountants' *International Code of Ethics for Professional Accountants (including International Independence Standards)*.

PricewaterhouseCoopers Inc., 4 Lisbon Lane, Waterfall City, Jukskei View, 2090
Private Bag X36, Sunninghill, 2157, South Africa
T: +27 (0) 11 797 4000, F: +27 (0) 11 209 5800, www.pwc.co.za

Chief Executive Officer: L S Machaba

The Company's principal place of business is at 4 Lisbon Lane, Waterfall City, Jukskei View, where a list of directors' names is available for inspection.
Reg. no. 1998/012055/21, VAT reg.no. 4950174682.

Other information

The directors are responsible for the other information. The other information comprises the information included in the document titled “Higher Education Training Health Wellness Development Centre NPC Annual Financial Statements for the year ended 31 March 2022”, which includes the Directors’ Report as required by the Companies Act of South Africa. The other information does not include the financial statements and our auditor’s report thereon.

Our opinion on the financial statements does not cover the other information and we do not express an audit opinion or any form of assurance conclusion thereon.

In connection with our audit of the financial statements, our responsibility is to read the other information identified above and, in doing so, consider whether the other information is materially inconsistent with the financial statements or our knowledge obtained in the audit, or otherwise appears to be materially misstated.

If, based on the work we have performed, we conclude that there is a material misstatement of this other information, we are required to report that fact. We have nothing to report in this regard.

Responsibilities of the directors for the financial statements

The directors are responsible for the preparation and fair presentation of the financial statements in accordance with International Financial Reporting Standards and the requirements of the Companies Act of South Africa, and for such internal control as the directors determine is necessary to enable the preparation of financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the financial statements, the directors are responsible for assessing the Company’s ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless the directors either intend to liquidate the Company or to cease operations, or have no realistic alternative but to do so.

Auditor’s responsibilities for the audit of the financial statements

Our objectives are to obtain reasonable assurance about whether the financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor’s report that includes our opinion. Reasonable assurance is a high level of assurance, but is not a guarantee that an audit conducted in accordance with ISAs will always detect a material misstatement when it exists. Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these financial statements.

As part of an audit in accordance with ISAs, we exercise professional judgement and maintain professional scepticism throughout the audit. We also:

- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and

obtain audit evidence that is sufficient and appropriate to provide a basis for our opinion. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.

- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Company's internal control.
- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by the directors.
- Conclude on the appropriateness of the directors' use of the going concern basis of accounting and, based on the audit evidence obtained, whether a material uncertainty exists related to events or conditions that may cast significant doubt on the Company's ability to continue as a going concern. If we conclude that a material uncertainty exists, we are required to draw attention in our auditor's report to the related disclosures in the financial statements or, if such disclosures are inadequate, to modify our opinion. Our conclusions are based on the audit evidence obtained up to the date of our auditor's report. However, future events or conditions may cause the Company to cease to continue as a going concern.
- Evaluate the overall presentation, structure and content of the financial statements, including the disclosures, and whether the financial statements represent the underlying transactions and events in a manner that achieves fair presentation.

We communicate with the directors regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that we identify during our audit.

PricewaterhouseCoopers Inc.

PricewaterhouseCoopers Inc.

Director: Raj Dhanlall

Registered Auditor

Waterfall City, Johannesburg, South Africa

3 March 2023

STATEMENT OF FINANCIAL POSITION

as at 31 March 2022

	2022 SA rands	2021 restated SA rands
ASSETS		
Non-current assets		
Property, plant and equipment	9 523 923	11 238 866
Current assets		
Inventories	4 396 741	–
Trade and other receivables	3 116 441	1 347 790
Cash and cash equivalents	55 662 414	43 889 431
	63 175 596	45 237 221
Total assets	72 699 519	56 476 087
EQUITY AND LIABILITIES		
Equity		
Accumulated surplus	9 651 578	7 251 045
Liabilities		
Current liabilities		
Trade and other payables	3 127 840	2 210 417
Finance lease liabilities	2 896 458	4 147 889
Deferred income	55 526 352	40 558 368
Provisions	1 497 291	2 308 368
	63 047 941	49 225 042
Total equity and liabilities	72 699 519	56 476 087

STATEMENT OF SURPLUS AND OTHER COMPREHENSIVE INCOME

for the year ended 31 March 2022

	2022 SA rands	2021 restated SA rands
Revenue	79 817 157	72 461 711
Cost of sales	(158 880)	–
Gross profit	79 658 277	72 461 711
Other income	(8 315)	5 555 478
Operating expenses	(76 977 455)	(74 155 029)
Surplus from operating activities	2 672 507	3 862 160
Finance costs	(271 974)	(350 735)
Surplus for the year	2 400 533	3 511 425

STATEMENT OF CHANGES IN EQUITY

for the year ended 31 March 2022

	Accumulated surplus SA rands	Total equity SA rands
Restated balance as at 1 April 2020	3 739 620	3 739 620
Profit for the year	3 511 425	3 511 425
Restated balance as at 1 April 2021	7 251 045	7 251 045
Profit for the year	2 400 533	2 400 533
Balance at 31 March 2022	9 651 578	9 651 578

NOTES

[illegible]

NOTES

[illegible]



HIGHER HEALTH

Higher Education & Training: Health, Wellness and Development Centre

📍 Wild Olive Building, 262 Rose Avenue,
Riverfalls Office Park, Centurion, Pretoria

☎ +27 (0)12 030 0113

✉ info@higherhealth.ac.za

www.higherhealth.ac.za